

Substance-Related Disorders - Self-Care

MCG Health
Chronic Care
28th Edition

CCG: C-0082 (CCG)

[Link to Codes](#)

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Assessment

Initial Consideration

Q1. Patient: What would you like to talk about today? (Check all that apply.) **QM**

- Emergency or urgent problems (eg, social determinants of health, intimate partner violence, suicidal ideation)^[A] Social Determinants of Health Screening Assessment [CCG](#) Intimate Partner Violence [CCG](#) Suicide or Personal Harm Risk [CCG](#)
- Condition information
- Provider coordination (eg, next appointment, treatment clarification) Appointment Scheduling [CCG](#)
- Medications
- Complications
- Monitoring and management
- Other health problems (eg, comorbid conditions, new symptoms)
- Other, specify: _____
- Nothing
- Unknown

Q2. Patient: Do you have any changes since the last time we talked? (Check all that apply.) **QM**

- Adhering to treatment or medication problems Readiness to Change [CCG](#) Medication Adherence [CCG](#)
- Administering medication Home Infusion Management [CCG](#) Injectable Medications [CCG](#)
- Affording medicine or supplies Financial Status and Benefits [CCG](#)
- Aspiration Aspiration Risk Management [CCG](#)
- Behavioral health provider appointment, specify: _____ Follow-Up - Provider Appointment [CCG](#)
- Blood pressure care Blood Pressure Monitoring [CCG](#)
- Blood sugar care Blood Glucose Monitoring [CCG](#) HbA1c Monitoring [CCG](#)
- Blood thinner care Anticoagulation Management [CCG](#)
- Bowel care Bowel Management [CCG](#)
- Bowel changes Bowel Symptoms [CCG](#)
- Caregiver Caregiver Resources Evaluation [CCG](#)
- Chemotherapy Chemotherapy [CCG](#)
- Cognitive impairment Cognitive Impairment [CCG](#) Behavioral Health and Cognition Evaluation with Caregiver [CCG](#)

- Communication or speech problems [Speech and Communication Problems](#)  CCG
- Community resource [Community Resources](#)  CCG
- Dehydration care [Dehydration Monitoring and Management](#)  CCG
- Depression symptoms [Depression Screening, Adult](#)  CCG
- Dialysis care [Dialysis Management](#)  CCG
- Doctor or specialist outpatient appointment, specify: _____ [Follow-Up - Provider Appointment](#)  CCG
- Energy level changes [Energy Level Changes](#)  CCG
- Enteral nutrition care [Enteral Nutrition Management](#)  CCG
- Exercising and staying active [Exercise](#)  CCG
- Fatigue or weakness [Fatigue or Weakness](#)  CCG
- Fluid restriction [Fluid Restriction](#)  CCG
- Functional status changes [Activities of Daily Living](#)  CCG [Instrumental Activities of Daily Living](#)  CCG
- Home infusion [Home Infusion Management](#)  CCG
- Hospital or emergency room visit, specify: _____ [Hospitalization Risk Assessment](#) 
 [CCG Post-Hospitalization Follow-Up](#)  CCG
- Medication changes [Medication Changes](#)  CCG
- Nebulizer care [Nebulizer Management](#)  CCG
- Neurologic function changes [Neurologic Function Changes](#)  CCG
- Ostomy care [Ostomy Management](#)  CCG
- Oxygen care [Oxygen Management](#)  CCG
- Pain, chronic [Chronic Pain](#)  CCG
- Pain, new or general [Pain Assessment](#)  CCG
- Parenteral nutrition care [Parenteral Nutrition Management](#)  CCG
- Peak flow meter care [Peak Flow Meter Use](#)  CCG
- Peripheral vascular (eg, lower leg redness or swelling) [Peripheral Vascular Problems](#)  CCG
- Physical impairments [Physical Impairments](#)  CCG
- Planning for long-term care needs [Advance Care Planning](#)  CCG
- Procedure, specify: _____ [Follow-Up - Provider Appointment](#)  CCG
- Pulmonary treatments [Pulmonary Treatments](#)  CCG
- Rehabilitation therapy, specify: _____ [Follow-Up - Provider Appointment](#)  CCG
- Respiratory changes [Respiratory Symptoms](#)  CCG
- Safety [Safety](#)  CCG
- Scheduling care [Appointment Scheduling](#)  CCG
- Sleeping problems [Sleep](#)  CCG
- Social support [Social Support](#)  CCG
- Stasis ulcer care [Stasis Ulcer Management](#)  CCG
- Testing (eg, laboratory, imaging), specify: _____ [Follow-Up - Provider Appointment](#) 
 CCG
- Tracheostomy care [Tracheostomy Management](#)  CCG
- Transportation [Transportation](#)  CCG
- Understanding healthcare needs [Health Literacy](#)  CCG
- Urinary catheter care [Urinary Catheter Management](#)  CCG
- Urinary changes [Urinary Symptoms](#)  CCG
- Venous thromboembolism symptoms [Venous Thromboembolism](#)  CCG
- Ventilator care [Ventilator Management](#)  CCG
- Weight gain [Nutritional Status, Adult](#)  CCG [Nutritional Status, Pediatric](#)  CCG [Nutritional Status, Infant](#)  CCG
- Weight loss [Nutritional Status, Adult](#)  CCG [Nutritional Status, Pediatric](#)  CCG [Nutritional Status, Infant](#)  CCG

- Wound care Wound Management  CCG
- Other, specify: _____
- No
- Unknown Hospitalization Risk Assessment  CCG

Condition Knowledge

Q1. Patient: What causes substance-related disorders? (Check items understood.)

- Cause unknown
- Chronic pain
- Disability or reduction in mobility
- Emotion control
- Family history (genetics)
- History of substance use in home
- Increasing amount or dosage of prescription drugs without provider approval
- Learned behavior
- Life-changing event (eg, death of a loved one, chronic condition diagnosis)
- Other physical or behavioral health problems
- Peer influence
- Rewarding behavior (physical and emotional response)
- Stress
- Traumatic experience in childhood
- Use for recreational purposes (seeking sense of feeling good, thrill-seeking, fantasy-seeking)
- Other, specify: _____
- Unknown [P1]
- Nothing [P1]
- Not all appropriate information known [P1]

Q2. Case Manager: Rate the patient's understanding of the causes of substance-related disorders. **QM**

- 90% or more
- 75% to 89% [P1]
- 50% to 74% [P1]
- Less than 50% [P1]
- Unknown [P1]

Q3. Patient: What are the symptoms of substance-related disorders? (Check items understood.)

- Abusive behavior
- Behaving fearfully, anxiously, or suspiciously
- Bloodshot eyes, pupils larger or smaller than usual
- Changes in eating habits
- Drug craving
- Getting into trouble frequently (fights, accidents, illegal activities)
- Memory impairment (blackouts)
- Neglect of family obligations
- Poor hygiene
- Poor muscle control
- Poor school performance
- Poor work performance
- Risky behavior (eg, using dirty needles, having unprotected sex, driving or operating heavy machinery under the influence)
- Skipping work or school
- Slurred speech
- Social withdrawal
- Sudden mood swings
- Sudden weight loss or gain
- Suspensions or expulsions from school
- Tremors
- Other, specify: _____
- Unknown [P1]
- Nothing [P1]
- Not all appropriate information known [P1]

Q4. Case Manager: Rate the patient's understanding of the symptoms of substance-related disorders. **QM**

- 90% or more

- 75% to 89% [P1]
- 50% to 74% [P1]
- Less than 50% [P1]
- Unknown [P1]

Q5. Patient: How can you care for your substance-related disorder to keep it from getting worse and causing problems? (Check items understood.) **QM**

- Adhere to self-care plan
- Ask for help when feelings are overwhelming
- Attend support group
- Exercise
- Follow a daily care plan
- Follow an eating (diet) plan
- Get all follow-up laboratory tests when due
- Keep a diary about feelings, activities, eating times, and medicines taken
- Keep all doctor and counseling appointments
- Keep a vaccination schedule
- Know when to contact the doctor
- Manage situation or event triggers
- Manage stress
- Seek emergency care for signs of crisis plan not working
- Seek treatment for mood disorders
- Set realistic expectations for feeling better, managing symptoms, and recovery
- Stop using alcohol, tobacco, or drugs
- Take medicines as ordered
- Other, specify: _____
- Unknown [P1]
- Nothing [P1]
- Not all appropriate information known [P1]

Q6. Case Manager: Rate the patient's understanding of how to care for substance-related disorder. **QM**

- 90% or more
- 75% to 89% [P1] [P2]
- 50% to 74% [P1] [P2]
- Less than 50% [P1] [P2]
- Unknown [P1] [P2]

Q7. Patient: What activities would you be able to do if you didn't have problems with your substance-related disorder? (Check all that apply.) **QM**

- Activities of daily living (ADL) without assistance [P2]
- Cook [P2]
- Drive [P2]
- Exercise [P2]
- Go out with friends [P2]
- Go to events [P2]
- Go to school [P2]
- Live independently [P2]
- Play with children or grandchildren [P2]
- Repair broken relationships [P2]
- Travel [P2]
- Work [P2]
- Other, specify: _____ [P2]
- Unknown [P2]

Provider Information

Q1. Patient: What doctor manages your substance-related disorder? **QM**

- Psychiatrist or psychologist, name and phone number: _____ [Q2]
- Behavioral health therapist, name and phone number: _____ [Q3]
- Primary doctor, name and phone number: _____ [P1]
- Other specialist, name and phone number: _____ [Q6] [P1]
- Don't have doctor [P1]

- Unknown [P1]

Q2. Patient: When was your last visit with the psychiatrist or psychologist?

- Less than 1 month ago
- 1 to 3 months ago [P2]
- 4 to 6 months ago [P2]
- More than 6 months ago [P2]
- Unknown [P2]
- Never [P2]
- Not applicable

Q3. Patient: When was your last visit with a behavioral health therapist?

- Less than 1 month ago
- 1 to 3 months ago [P2]
- 4 to 6 months ago [P2]
- More than 6 months ago [P2]
- Unknown [P2]
- Never [P2]
- Not applicable

Q4. Patient: When was your last visit with your primary doctor? **QM**

- Within the past 3 months
- 4 to 6 months ago [P2]
- More than 6 months ago [P2]
- Unknown [P2]
- Never [P2]

Q5. Patient: What other specialists have you seen within the last 12 months? (Check all that apply.) **QM**

- Behavioral health provider
- Cardiologist
- Dentist
- Dietitian
- Endocrinologist
- Gastroenterologist
- General surgeon
- Genetic counselor
- Geriatrician
- Gynecologist
- Hematologist
- Hepatologist
- HIV/AIDS
- Infectious disease
- Nephrologist
- Neurologist
- Neurosurgeon
- Obstetrician
- Oncologist
- Ophthalmologist
- Orthopedic surgeon
- Orthopedist
- Otolaryngologist
- Pain management
- Psychiatrist
- Plastic surgeon
- Podiatrist
- Pulmonologist
- Rheumatologist
- Transplant specialist
- Urologist
- Other, specify: _____
- None [P1]
- Unknown [P1]

Q6. Patient: When was your last visit to the specialist doctor? (Specify specialist type.)

- Within the past 4 months, specify: _____
- 4 to 6 months ago, specify: _____ [P2]
- More than 6 months ago, specify: _____ [P2]
- Unknown [P2]
- Never [P2]
- Not applicable

Q7. Patient: How many substance use-related emergency department visits or hospital admissions have you had in the past 12 months? **QM**

- 2 or more [P2] Hospitalization Risk Assessment [CCG](#)
- 1 [P2]
- None
- Other, specify: _____
- Unknown [P2]

Q8. Patient: Do you have a written self-care plan from your doctor on how to care for your substance-related disorder?[B] **QM**

- Yes [Q9]
- No [P1] [P2]
- Unknown [P1] [P2]

Q9. Patient: What are the components of your substance-related disorder self-care plan? (Check all that apply.) **QM**

- Action plan for when cravings start
- Attending regular therapy or counseling appointments
- Attending support groups
- Crisis management plan
- Exercise plan
- Knowing what medicines you take and why and how to take them
- Managing side effects of medicines
- Medicine use
- Nutrition plan
- Recognizing the symptoms of gradual worsening or problems with condition
- Regular follow-up appointments for wellness checks
- Stress management
- When to call the doctor and when to get emergency help
- Other, specify: _____ [P1]
- Unknown [P2]
- Not applicable
- Not all appropriate information known [P2]

Q10. Case Manager: Is the patient able to understand and adhere to the self-care plan? **QM**

- Yes, independently
- Yes, with occasional reminders or caregiver support
- Yes, with frequent reminders or caregiver support
- No [P3]
- Unknown [P3]

Medication Information

Q1. Patient: Has any medicine for substance-related disorder been prescribed by your doctor? If so, is it taken as ordered? **QM**

- Yes, and taken as ordered [Q2]
- Yes, but not taken as ordered [Q2] [P4] Medication Adherence [CCG](#)
- No medicine prescribed [P4]
- Unknown [P4]

Q2. Patient: What medicines do you take for substance-related disorder withdrawal and relapse prevention? (Check all that apply.) **QM**

- Acamprosate (alcohol) [Q3] [P5] Alcohol Misuse [CCG](#)
- Anticonvulsants (alcoholic withdrawal seizures) (eg, carbamazepine, gabapentin, topiramate, valproic acid) [Q3] [P5] Alcohol Misuse [CCG](#)
- Benzodiazepines (alcohol) (eg, chlordiazepoxide, diazepam, lorazepam) [Q3] [P5] Alcohol Misuse [CCG](#)

- Disulfiram (alcohol) [Q3] [P5] Alcohol Misuse [CCG](#)
- Opioid agonists (eg, buprenorphine/naltrexone combination, methadone) [Q3] [P5] Narcotic/Opioid Misuse Assessment [CCG](#)
- Opioid antagonist (eg, naltrexone) [Q3] [P5] Narcotic/Opioid Misuse Assessment [CCG](#)
- Pain medicine, over-the-counter (eg, acetaminophen, nonsteroidal anti-inflammatory drugs (NSAIDs)) [P5] Pain Medication Use [CCG Pain Assessment](#) [CCG](#)
- Pain medicine, prescription (eg, narcotics, opioids) [P5] Pain Medication Use [CCG Pain Assessment](#) [CCG](#)
- Other, specify: _____ [Q3] [P4] [P5]
- Not applicable [P4]
- Unknown [P5]

Q3. Patient: Are you having any side effects from the substance-related disorder medicine? (Check all that apply.)

- Agitation [P5]
- Blurred vision [P5]
- Confusion [P5]
- Dark or hard stools (constipation) [P5]
- Depression [P5]
- Dizziness [P5]
- Drowsiness [P5]
- Dry mouth [P5]
- Fast heartbeats [P5]
- Feeling sick to your stomach (nausea) or throwing up (vomiting) [P5]
- Headache [P5]
- Inability to concentrate [P5]
- Inability to sleep [P5]
- Loss of muscle coordination [P5]
- Not hungry [P5]
- Sexual problems (abnormal ejaculation, impotence, or loss of orgasm) [P3]
- Tiredness [P5]
- Other, specify: _____ [P5]
- None
- Unknown [P5]
- Not applicable

Q4. Case Manager: Rate the patient's understanding of the purpose and side effects of the medications for substance-related disorder. **QM**

- 90% or more
- 75% to 89% [P1] [P5]
- 50% to 74% [P1] [P5]
- Less than 50% [P1] [P5]
- Unknown [P1] [P5]

Q5. Patient: Do you take any medicine that may affect medicines you take for withdrawal or to maintain sobriety or stay clean? (Check all that apply.)

- Antibiotics [P3] [P4]
- Anticonvulsants [P3] [P4]
- Antidepressants [P3] [P4]
- Antivirals [P3] [P4]
- Benzodiazepines [P3] [P4]
- Phenytoin [P3] [P4]
- Other, specify: _____ [P3] [P4]
- None
- Unknown [P4]

Complications

Q1. Patient: Right now, do you have thoughts of suicide or harming yourself or someone else? **QM**

- Yes [B1] [P6]
- No
- Unknown [B1] [P6]

Q2. Patient: Have you had any of the following symptoms over the past 2 weeks? (Check all that apply.)

- Becoming frustrated or losing your temper easily [P7]
- Crying spells for no reason [P7]
- Eating too much or not wanting to eat at all [P7]
- Feeling guilty [P7]
- Feeling sad or unhappy a lot [P7]
- Feeling tired, sluggish, or physically drained [P7]
- Feeling worthless or hopeless [P7]
- Having trouble sleeping or sleeping too much [P7]
- Little or no interest in activities or hobbies that you enjoy, including sex [P7]
- Little or no interest in being with your friends and family [P7]
- Poor personal hygiene, not wanting to bathe, or not caring about what you wear [P7]
- Thoughts of death, suicide, or harming yourself or someone else [P7]
- Trouble concentrating or remembering things [P7]
- Trouble making decisions [P7]
- None
- Unknown [P7]

Q3. Patient: Have you ever tried to commit suicide? **QM**

- Yes [P7]
- No
- Unknown

Q4. Patient: Answer the next 11 questions as you have felt over the past 2 weeks.(1) Have you lost interest in doing things you used to enjoy?  PHQ-9 Calculator **QM**

- No
- Sometimes [P8]
- Most days (more than 1/2 of days in past 2 weeks) [P8]
- Almost every day [P8]
- Every day [P8]
- Unknown [P8]

Q5. Patient: Do you feel hopeless for the future? **QM**

- No
- Sometimes [P8]
- Most days (more than 1/2 of days in past 2 weeks) [P8]
- Almost every day [P8]
- Every day [P8]
- Unknown [P8]

Q6. Patient: Do you have trouble concentrating (such as when reading or following what people tell you)? **QM**

- No
- Sometimes [P8]
- Most days (more than 1/2 of days in past 2 weeks) [P8]
- Almost every day [P8]
- Every day [P8]
- Unknown [P8]

Q7. Patient: Do you feel you have no pleasure or happiness in your life? **QM**

- No
- Sometimes [P8]
- Most days (more than 1/2 of days in past 2 weeks) [P8]
- Almost every day [P8]
- Every day [P8]
- Unknown [P8]

Q8. Patient: Are you easily agitated, angry, or upset by things? **QM**

- No
- Sometimes [P8]
- Most days (more than 1/2 of days in past 2 weeks) [P8]
- Almost every day [P8]

- Every day [P8]
- Unknown [P8]

Q9. Patient: Does it seem to take a lot of effort to do simple things? **QM**

- No
- Sometimes [P8]
- Most days (more than 1/2 of days in past 2 weeks) [P8]
- Almost every day [P8]
- Every day [P8]
- Unknown [P8]

Q10. Patient: Are you having problems making decisions? **QM**

- No
- Sometimes [P8]
- Most days (more than 1/2 of days in past 2 weeks) [P8]
- Almost every day [P8]
- Every day [P8]
- Unknown [P8]

Q11. Patient: Have you lost or gained weight without trying? **QM**

- Yes [P8]
- No
- Unknown [P8]

Q12. Patient: Has your sleep pattern changed, such as sleeping more or less than usual? **QM**

- Yes [P8]
- No
- Unknown [P8]

Q13. Patient: Have you had a life-changing event in the past 6 months? (Check all that apply.) **QM**

- Death of someone close [P8]
- Divorce [P8]
- Had a child [P8]
- Lost a job or changed jobs [P8]
- Major illness or injury [P8]
- Moved [P8]
- Other, specify: _____ [P8]
- None
- Unknown

Q14. Patient: Have any of these problems made it hard for you to work, care for yourself, take care of things at home, or get along with other people? **QM**

- No
- Sometimes [P8]
- Most days (more than 1/2 of days in past 2 weeks) [P8]
- Almost every day [P8]
- Every day [P8]
- Unknown [P8]

Q15. Patient: What are some problems you can have due to using substances? (Check items understood.)

- Changes in behavior at work (eg, not getting your work done on time, problems with coworkers, late to work a lot, or not going to work)
- Changes in behavior in school (eg, being late to school a lot, not going to school, changes in your grades in classes, or fighting)
- Decreased ability to think or concentrate
- Decreased desire for sexual activity
- Legal problems like being arrested, stealing, or financial difficulties
- Lost employment
- Not feeling like eating or overeating
- Suspension or expulsion from school
- Other, specify: _____
- None [P1]
- Unknown [P1]

- Not all appropriate information known [P1]

Q16. Patient: What problems with substance use have you had in the past month? (Check all that apply.)

- Changes in behavior at work (eg, not getting your work done on time, problems with coworkers, late to work a lot, or not going to work) [P2] [P3]
- Changes in behavior in school (eg, being late to school a lot, not going to school, changes in your grades in classes, or fighting) [P2] [P3]
- Decreased ability to think or concentrate [P2] [P3]
- Decreased desire for sexual activity [P2] [P3]
- Legal problems like being arrested, stealing, or financial difficulties [P2] [P3]
- Lost employment [P2] [P3]
- Not feeling like eating or overeating [P2] [P3]
- Suspension or expulsion from school [P2] [P3]
- Other, specify: _____ [P2] [P3]
- None
- Unknown [P2] [P3]

Q17. Case Manager: Rate the patient's understanding of the potential complications of substance-related disorder. **QM**

- 90% or more
- 75% to 89% [P1] [P2]
- 50% to 74% [P1] [P2]
- Less than 50% [P1] [P2]
- Unknown [P1] [P2]

Monitoring and Management

Q1. Patient: In the past 4 weeks, which of these substances have you used? Please specify how many times the substance was used. (Check all that apply.) **QM**

- ADHD drugs not prescribed for a medical condition, frequency of use: _____ [P9]
- ADHD drugs used more than prescribed, frequency of use: _____ [P9]
- Amphetamines, frequency of use: _____ [P9]
- Anxiety drugs not prescribed for a medical condition, frequency of use: _____ [P9]
- Anxiety drugs used more than prescribed, frequency of use: _____ [P9]
- Cannabinoids, synthetic, frequency of use: _____ [P9]
- Cathinones, synthetic, frequency of use: _____ [P9]
- Cocaine, frequency of use: _____ [P9]
- Desomorphine, frequency of use: _____ [P9]
- Flunitrazepam (Rohypnol), frequency of use: _____ [P9]
- Hallucinogens, such as LSD (lysergic acid diethylamide), mescaline (peyote), or psilocybin (mushrooms), frequency of use: _____ [P9]
- Heroin, frequency of use: _____ [P9]
- Inhalants (huffing), frequency of use: _____ [P9]
- Insomnia drugs not prescribed for a medical condition, frequency of use: _____ [P9]
- Insomnia drugs used more than prescribed, frequency of use: _____ [P9]
- Kratom, frequency of use: _____ [P9]
- Marijuana (cannabis) not prescribed for a medical condition, frequency of use: _____ [P9]
- Marijuana (cannabis) used more than prescribed, frequency of use: _____ [P9]
- MDMA (3,4-methylenedioxymethamphetamine (ecstasy)), frequency of use: _____ [P9]
- Methamphetamine, frequency of use: _____ [P9]
- Narcotic drugs not prescribed for a medical condition, frequency of use: _____ [P9]
- Narcotic/Opioid Misuse Assessment [CCG](#)
- Narcotic drugs prescribed for medical condition, frequency of use: _____ [P9]
- Narcotic/Opioid Misuse Assessment [CCG](#)
- PCP (phencyclidine), frequency of use: _____ [P9]
- Salvia, frequency of use: _____ [P9]
- Steroids, frequency of use: _____ [P9]
- Other, specify: _____ [P9]
- None
- Unknown [P9]

Q2. Patient: How long have you been using illegal substances or misusing legal drugs (drugs that are not prescribed for you or not taken as ordered)? **QM**

- Never have used illegal substances or misused legal drugs
- Less than 1 year [Q3] [P10]
- 1 to 3 years [Q3] [P10]
- Longer than 3 years [Q3] [P10]
- Unknown [Q3] [P9]

Q3. Patient: When was the last time you were high?

- Specify: _____ [P9]
- Unknown [P9]

Q4. Patient: How often did you have a drink with alcohol in the past year?[C] **QM**

- Never
- Monthly or less often [Q5]
- 2 to 4 times per month [Q5]
- 2 to 3 times per week [Q5]
- 4 or more times per week [Q5]
- Unknown

Q5. Patient: How many drinks did you have on a typical day in the past year? **QM**

- None
- 1 to 2
- 3 to 4 [P11] Alcohol Misuse [CCG](#)
- 5 to 6 [P11] Alcohol Misuse [CCG](#)
- 7 to 9 [P11] Alcohol Misuse [CCG](#)
- 10 or more [P11] Alcohol Misuse [CCG](#)
- Unknown

Q6. Patient: How often did you have 6 or more drinks on one occasion in the past year? **QM**

- Never
- Less than monthly
- Monthly [P11] Alcohol Misuse [CCG](#)
- Weekly [P11] Alcohol Misuse [CCG](#)
- Daily or almost daily [P11] Alcohol Misuse [CCG](#)

Q7. Patient: Do you have any risk factors for alcohol misuse? (Check all that apply.) **QM**

- Comorbid behavioral health diagnosis (eg, depression, posttraumatic stress disorder) Alcohol Misuse [CCG](#)
- Family history Alcohol Misuse [CCG](#)
- Life-changing event (eg, death of a loved one, chronic condition diagnosis) Alcohol Misuse [CCG](#)
- Traumatic experience in childhood Alcohol Misuse [CCG](#)
- Other, specify: _____ Alcohol Misuse [CCG](#)
- None
- Unknown

Q8. Patient: Are you interested in treatment for your substance-related disorder?

- Yes [P2]
- No Readiness to Change [CCG](#)
- Unknown [P2]

Q9. Patient: Have you participated in a substance-related disorder treatment program? If so, when?

- Currently participating, specify program: _____ [Q10]
- Yes, 3 months ago or less
- Yes, 4 to 11 months ago
- Yes, 12 months ago or more [P2]
- Never [P2]
- Unknown [P2]
- Not applicable

Q10. Patient: What type of program are you in for treating substance-related disorders? (Check all that apply.)

- Community education program
- Exercise program
- Inpatient rehabilitation program
- Medication replacement (eg, methadone)
- Outpatient rehabilitation program
- Support group
- None now, but in the past, specify: _____ [P2] [P3]
- Other, specify: _____
- None [P2] [P3]
- Unknown [P2] [P3]

Q11. Patient: Is there someone you talk to about your problems?

- Yes, name and relationship: _____
- No [B2] [P12]
- Unknown [B2] [P12]

Q12. Patient: Do you have someone to count on when you need help?

- Yes, name and relationship: _____
- No [B2] [P12]
- Unknown [B2] [P12]

Q13. Patient: Is there someone who can help you feel better when you are stressed, sad, or upset?

- Yes, name and relationship: _____
- No [B2] [P12]
- Unknown [B2] [P12]

Q14. Patient: Is there someone who accepts you for who you are, the good and bad?

- Yes, name and relationship: _____
- No [B2] [P12]
- Unknown [B2] [P12]

Q15. Patient: Is there someone you can count on to care about you no matter what is happening?

- Yes, name and relationship: _____
- No [B2] [P12]
- Unknown [B2] [P12]

Q16. Patient: Does your social support help you as much as you need? **QM**

- Yes
- No [B2] [P12]
- Unknown [B2] [P12]

Q17. Patient: How much do you weigh and how tall are you? **QM**

- Weight: _____
- Height: _____
- Unknown [P13]

Q18. Case Manager: What is the patient's body mass index (BMI)?  BMI Calculator

- Less than 18.5 [P13] [P14] Nutritional Management to Gain Weight  CCG Body Weight Planner  CCG
- 18.5 to 24.9
- 25 to 29.9 [P13] [P14] Nutritional Management to Lose Weight  CCG Body Weight Planner  CCG
- 30 or greater [P13] [P14] Nutritional Management to Lose Weight  CCG Body Weight Planner  CCG
- Unknown [P13]

Q19. Patient: Do you know what your body weight should be?

- Yes, it should be: _____
- No [P13]
- Unknown [P13]

Q20. Patient: What do you eat and drink in a typical day? (Check all reported.)

- Alcohol, specify types and number of daily servings: _____ [Q21] [Q22] [Q23] [Q24] [Q25]

- Dairy foods, specify types and number of daily servings: _____ [Q21] [Q22] [Q23] [Q24] [Q25]
- Fats, specify types and number of daily servings: _____ [Q21] [Q22] [Q23] [Q24] [Q25]
- Fruits, specify types and number of daily servings: _____ [Q21] [Q22] [Q23] [Q24] [Q25]
- Fruit juices, specify types and number of daily servings: _____ [Q21] [Q22] [Q23] [Q24] [Q25]
- Grains, specify types and number of daily servings: _____ [Q21] [Q22] [Q23] [Q24] [Q25]
- Meats (beef, chicken, fish), specify types and number of daily servings: _____ [Q21] [Q22] [Q23] [Q24] [Q25]
- Milk, specify types and number of daily servings: _____ [Q21] [Q22] [Q23] [Q24] [Q25]
- Sodas, specify types and number of daily servings: _____ [Q21] [Q22] [Q23] [Q24] [Q25]
- Supplements, specify types and number of daily servings: _____ [Q21] [Q22] [Q23] [Q24] [Q25]
- Sweets, specify types and number of daily servings: _____ [Q21] [Q22] [Q23] [Q24] [Q25]
- Vegetables, specify types and number of daily servings: _____ [Q21] [Q22] [Q23] [Q24] [Q25]
- Water, specify number of daily servings: _____ [Q21] [Q22] [Q23] [Q24] [Q25]
- Other, specify: _____ [Q21] [Q22] [Q23] [Q24] [Q25] [P13]
- No food by mouth [Q25] [P13] [P14]
- Unknown [Q21] [Q22] [Q23] [Q24] [Q25] [P13] [P14]

Q21. Patient: How often do you eat each day? (Check all reported.) **QM**

- 3 meals or more [P13] [P14]
- 2 meals
- 1 meal [P13] [P14]
- 0 meals [P13] [P14]
- 2 snacks or more [P13] [P14]
- 1 snack
- 0 snacks
- Other, specify: _____ [P13] [P14]
- Not applicable
- Unknown [P13] [P14]

Q22. Patient: How much fluid (such as water, juice, coffee, tea, milk) do you have per day? **QM**

- Less than 3 cups (720 mL) [P13] [P14]
- 3 to 5 cups (720 mL to 1.2 L) [P13] [P14]
- More than 5 cups (1.2 L)
- Unknown [P13] [P14]

Q23. Patient: Are you trying to gain or lose weight?

- Trying to gain weight [P13] [Nutritional Management to Gain Weight](#) [CCG Body Weight Planner](#) [CCG](#)
- Trying to lose weight [P13] [Nutritional Management to Lose Weight](#) [CCG Body Weight Planner](#) [CCG](#)
- Neither
- Unknown [P13]

Q24. Patient: Does your doctor want you to follow a special diet program? (Check all that apply.) **QM**

- Yes, DASH (low sodium) [P13] [Diet Knowledge Insufficient - DASH Diet](#) [CCG](#)
- Yes, heart healthy [P13] [Diet Knowledge Insufficient - Heart Healthy](#) [CCG](#)
- Yes, kidney diet [P13] [Diet Knowledge Insufficient - Renal Dysfunction](#) [CCG](#)
- Yes, low carb or low glycemic index [P13] [Diet Knowledge Insufficient - Low Glycemic Index](#) [CCG](#)
- Yes, pregnancy diet [P13] [Diet Knowledge Insufficient - Pregnancy](#) [CCG](#)
- Yes, other: _____ [P13]
- No [P13]
- Unknown [P13]

Q25. Patient: Do you get nutritional treatments or have an altered diet? If yes, why? (Check all that apply.) **QM**

- Behavioral health provider seen, specify: _____ [P14]
- Dental soft diet, specify: _____ [P14]
- Diet supplements, specify: _____ [P14]

- Dietitian seen or following, specify: _____ [P14]
- Enteral nutrition (tube feeding), specify: _____ [P14] [Enteral Nutrition Management](#) 
- IV fluids (hydration), specify: _____ [P14]
- Liquid diet, specify: _____ [P14]
- Mechanical soft diet, specify: _____ [P14]
- Parenteral nutrition, specify: _____ [P14] [Parenteral Nutrition Management](#)  CCG
- Procedure(s) done by a doctor in office, specify: _____ [P14]
- Pureed diet, specify: _____ [P14]
- Speech-language pathology (SLP), specify: _____ [P14]
- Supervised during nutritional intake, specify: _____ [P14]
- Surgery, specify: _____ [P14]
- Thickened liquid diet, specify: _____ [P14]
- Other, specify: _____ [P14]
- No
- Unknown [P13]

Q26. Patient: Do you know how to read food labels?

- Yes
- No [P15]
- Unknown [P15]

Q27. Patient: Do you get regular exercise? **QM**

- Yes
- No [P2]
- Unknown [P2]

Q28. Patient: Are you having any problems with your substance-related disorder self-care needs? (Check all that apply.) **QM**

- Affording medicine or supplies [P2] [P3] [Financial Status and Benefits](#)  CCG
- Behavior indicating cognitive impairment [P2] [P3] [Cognitive Impairment](#)  CCG [Behavioral Health and Cognition Evaluation with Caregiver](#)  CCG
- Caregiver help needed [P2] [P3] [Caregiver Resources Evaluation](#)  CCG
- Chronic pain management [P2] [P3] [Chronic Pain](#)  CCG
- Community resource information needed [P2] [P3] [Community Resources](#)  CCG
- Daily care of child [P3] [Caregiver Resources Evaluation](#)  CCG
- Exercising and staying active [P2] [P3] [Exercise](#)  CCG
- Safety in the home [P2] [P3] [Safety](#)  CCG
- Sleeping problems [P2] [P3] [Sleep](#)  CCG
- Social support [P2] [P3] [Social Support](#)  CCG
- Taking medicine as prescribed [P2] [P3] [Medication Adherence](#)  CCG
- Transportation for care [P2] [P3] [Transportation](#)  CCG
- Trouble gaining weight [P2] [P3] [Nutritional Management to Gain Weight](#)  CCG
- Trouble losing weight [P2] [P3] [Nutritional Management to Lose Weight](#)  CCG
- Trouble making lifestyle changes [P2] [P3] [Readiness to Change](#)  CCG
- Trouble understanding how to do self-care [P2] [P3] [Health Literacy](#)  CCG
- Other, specify: _____ [P2] [P3]
- No
- Unknown [P2] [P3]

Q29. Case Manager: Rate the patient's understanding of the monitoring and management of substance-related disorder. **QM**

- 90% or more
- 75% to 89% [P1] [P2]
- 50% to 74% [P1] [P2]
- Less than 50% [P1] [P2]
- Unknown [P1] [P2]

Other Health Considerations

Q1. Patient: Do you currently smoke or use tobacco products? (Check all that apply.) **QM**

- Yes, I smoke. [Q2] [Q4] [Q5] [Q6]
- Yes, I use other tobacco product; specify: _____ [Q3] [Q4] [Q5] [Q6]
- No, I quit. [Q4] [Q7]
- No, I have never smoked or used other tobacco products.
- Unknown

Q2. Patient: How much do you smoke?

- 2 packs a day or more [P16]
- More than 1 but less than 2 packs a day [P16]
- 1 pack a day [P16]
- Less than 1 pack a day [P16]
- Unknown
- Not applicable

Q3. Patient: How often do you use other tobacco products?

- 3 or more times a day [P16]
- 1 or 2 times a day [P16]
- A few times a week [P16]
- A few times a month [P16]
- A few times a year [P16]
- Unknown [Q6]
- Not applicable

Q4. Patient: How old were you when you started smoking or using tobacco products?

- Specify age: _____
- Unknown

Q5. Patient: What triggers you to want to smoke or use tobacco? (Check all that apply.)

- After meals [P16]
- Alcohol use [P16]
- Being around people who are smoking [P16]
- Driving or talking on the phone [P16]
- Stress [P16]
- Other, specify: _____ [P16]
- Nothing [P16]
- Unknown [P16]
- Not applicable

Q6. Patient: Are you ready to stop smoking or using tobacco products? **QM**

- Yes [P16]
- No [P16] Readiness to Change  CCG
- Unknown [P16] Readiness to Change  CCG

Q7. Patient: When did you quit smoking or using tobacco products?

- More than 1 year ago [P17]
- Within the last year [P17]
- Unknown

Q8. Patient: Are you pregnant or planning a pregnancy?

- Yes, pregnant [P2] [P3] High-Risk Pregnancy - Self-Care  CCG
- Yes, planning pregnancy [P2] [P3]
- No
- Unknown [P2] [P3]

Q9. Patient: What vaccinations have you had? (Check all that apply.) **QM**

- Coronavirus disease 2019 (COVID-19) [Q10]
- *Haemophilus influenzae* type b (Hib) [Q10] [Q11]
- Hepatitis A [Q10]
- Hepatitis B [Q10]

- Human papillomavirus (HPV) [Q10]
- Influenza [Q10] [Q11]
- Measles, mumps, rubella (MMR) [Q10]
- Meningococcal 4-valent conjugate (MenACWY) or polysaccharide (MPSV4) [Q10]
- Meningococcal B (MenB) [Q10]
- Pneumococcal 13-valent conjugate (PCV13) [Q10] [Q12]
- Pneumococcal 15-valent conjugate (PCV15) [Q10] [Q12]
- Pneumococcal 20-valent conjugate (PCV20) [Q10] [Q12]
- Pneumococcal 23-valent polysaccharide (PPSV23) [Q10] [Q12]
- Polio (inactivated) [Q10]
- Respiratory syncytial virus (RSV) [Q10]
- Rotavirus [Q10]
- Tetanus booster [Q10]
- Tetanus, diphtheria, pertussis (Td/Tdap) [Q10]
- Varicella [Q10]
- Zoster [Q10]
- Other, specify: _____ [Q10]
- None, because: _____ [P18]
- Unknown [P18]

Q10. Patient: Are you up to date with the vaccinations you need (for age, booster status, or medical condition)? **QM**

- Yes
- No, because: _____ [Q13] [P18]
- Unknown [Q13] [P18]

Q11. Patient: When was your last flu shot? **QM**

- In the past year
- More than a year ago [Q13] [P18]
- Never [Q13] [P18]
- Unknown [Q13] [P18]

Q12. Patient: When was your last pneumonia shot?

- In the last year
- 2 to 4 years ago
- 4 to 5 years ago [Q13] [P18]
- More than 5 years ago [Q13] [P18]
- Never [Q13] [P18]
- Unknown [Q13]

Q13. Patient: Do you have an appointment scheduled to get caught up on your vaccinations?

- Yes
- No [P18]
- Unknown [P18]

Q14. Patient: Do you have other conditions that might make substance-related disorders worse? (Check all that apply.) **QM**

- Anxiety [P2] [P3] Anxiety - Self-Care [CCG](#)
- Attention-deficit hyperactivity disorder [P2] [P3] Attention-Deficit Hyperactivity Disorder, Adult - Self-Care [CCG](#) Attention-Deficit Hyperactivity Disorder, Pediatric - Self-Care [CCG](#)
- Bipolar disorder [P2] [P3] Bipolar Disorder - Self-Care [CCG](#)
- Cancer [P2] [P3] Cancer - Self-Care [CCG](#)
- Chronic obstructive pulmonary disease (COPD) [P2] [P3] Chronic Obstructive Pulmonary Disease - Self-Care [CCG](#)
- Chronic pain management [P2] [P3] Chronic Pain [CCG](#)
- Cirrhosis [P2] [P3] Liver Disease - Self-Care [CCG](#)
- Coronary artery disease [P2] [P3] Coronary Artery Disease - Self-Care [CCG](#)
- Depression [P2] [P3] Depression - Self-Care [CCG](#)
- Diabetes [P2] [P3] Diabetes, Adult - Self-Care [CCG](#) Diabetes, Pediatric - Self-Care [CCG](#)
- Fetal alcohol syndrome [P2] [P3]
- Gout [P2] [P3] Arthritis - Self-Care [CCG](#)

- Heart failure [P2] [P3] Heart Failure - Self-Care  CCG
- Hepatitis B or C [P2] [P3] Liver Disease - Self-Care  CCG
- High blood pressure [P2] [P3] Hypertension - Self-Care  CCG
- High cholesterol [P2] [P3] High Cholesterol - Self-Care  CCG
- HIV/AIDS [P2] [P3] HIV/AIDS - Self-Care  CCG
- Kidney failure [P2] [P3] Kidney Disease - Self-Care  CCG
- Obesity or overweight [P2] [P3] Obesity, Adult - Self-Care  CCG Obesity, Pediatric - Self-Care  CCG
- Personality disorders [P2] [P3]
- Physical inactivity [P2] [P3] Exercise  CCG
- Posttraumatic stress disorder (PTSD) [P2] [P3] Anxiety - Self-Care  CCG
- Pregnancy [P2] [P3] High-Risk Pregnancy - Self-Care  CCG
- Schizophrenia [P2] [P3] Schizophrenia - Self-Care  CCG
- Tuberculosis [P2] [P3]
- Other, specify: _____ [P2] [P3]
- No
- Unknown [P1]

Patient Understanding Evaluation

Q1. Case Manager: Does the patient understand all the causes of substance-related disorder?

- Yes
- No [P1]
- Unknown [P1]

Q2. Case Manager: Does the patient understand all the symptoms of substance-related disorder?

- Yes
- No [P1]
- Unknown [P1]

Q3. Case Manager: Does the patient understand how to care for substance-related disorder? **QM**

- Yes
- No [P1] [P2]
- Unknown [P1] [P2]

Q4. Case Manager: Does the patient know when and where to get help for problems?

- Yes
- No [P1] [P3]
- Unknown [P1] [P3]

Q5. Case Manager: Does the patient have the ability to understand and follow the self-care plan? **QM**

- Yes
- No [P3]
- Unknown [P3]

Q6. Case Manager: Does the patient understand the purpose and side effects of their substance-related disorder medication?

- Yes
- No [P5]
- Unknown [P5]

Q7. Case Manager: Is the patient having trouble coping with their substance-related disorder? **QM**

- Yes [P3]
- No
- Unknown [P3]

Q8. Case Manager: Does the patient agree to education? **QM**

- Yes
- No, explain why: _____ [P3] Readiness to Change  CCG
- Unknown [P3]

Quality Measures

- Disease Management Quality Measures:
 - Information provided from this question may be requested by various accrediting organizations.
- HEDIS Measures include(2):
 - Tobacco use is assessed and addressed.
 - Readmission risk is assessed and addressed.
 - Alcohol and substance misuse is assessed and addressed.

Plan of Care

Barriers

B1. Personal harm risk, immediate

B2. Inadequate social support

Problems

P1. Substance-related disorders information needed

- **Goal:** Substance-related disorders diagnosis better understood(3)(4) 
- **Interventions:**
 - **ASSIST:** Identifying patient's goal priorities, willingness/ability to achieve goals, and preferences for communication and follow-up
 - **ASSIST:** Goal setting that is specific, measurable, achievable, relevant, and with target completion time (eg, short/long-term, prioritized)
 - **ASSIST:** Appointment scheduling
 - **EDUCATE: Definition(5):**
 - Disorders of intoxication, dependence, and misuse caused by various legal and illegal substances
 - Neurobiological aspect: Intoxicant exposure increases firing in the dopaminergic pathways of the ventral striatum, which gives a subjective high, promoting further drug use.(6)(7)
 - Chronic disorders may alter the area of the brain that affects judgment and behavior, driving compulsion to obtain and use substances, making abstinence difficult.
 - **EDUCATE: Causes:**
 - Family history (genetics)
 - Traumatic event history
 - Psychosocial and environmental influences
 - Medications prescribed for conditions (eg, opioids, sleep aids, stimulants) can lead to substance-related disorders.
 - **EDUCATE: Symptoms(5):**
 - Behavioral changes (eg, sudden mood swings, aggression, isolation)
 - Risky behaviors (eg, gambling, shopping sprees, promiscuity)(8)
 - Excessive time using substances
 - Tolerance to substance
 - Memory impairment (blackouts)
 - Tremors
 - Repeated episodes of intoxication
 - **EDUCATE: Diagnosis:**
 - Based on medical history, risk factors, physical examination, and diagnostic tests
 - Positive screening instrument
 - **EDUCATE: Treatment(8)(9):**
 - Psychotherapy (eg, cognitive behavioral therapy (CBT), individual or group therapy, family therapy)
 - Medication management[D]
 - Medications for pre-existing conditions may need to be changed (type or dose).
 - Comorbid condition management
 - Lifestyle changes (eg, nutrition, exercise, tobacco cessation)
 - Symptom management
 - **EDUCATE: Treatment goals:**
 - Psychosocial functioning improved and maintained
 - Quality of life improvement
 - Relapse reduction
 - **EDUCATE: Resources:**
 - Al-Anon Family Groups: www.al-anon.org
 - Alcoholics Anonymous World Services: www.aa.org
 - Cocaine Anonymous World Services: www.ca.org
 - Marijuana Anonymous World Services: www.marijuana-anonymous.org
 - Narcotics Anonymous World Services: www.na.org

- National Institute on Alcohol Abuse and Alcoholism: www.niaaa.nih.gov
- National Suicide Prevention Lifeline: www.988lifeline.org
- Smokefree. National Institutes of Health: www.smokefree.gov
- Substance Abuse and Mental Health Services Administration: www.samhsa.gov
- **COORDINATE:** Behavioral health services referral, if indicated(10)
- **COORDINATE:** Psychiatrist referral, if indicated(11)(12)
- **COORDINATE:** Laboratory testing, if indicated(13)
- **COORDINATE:** Social worker referral, as needed(14)
- **COORDINATE:** Support group referral, as needed
- **COORDINATE:** Care team meeting, if needed(15)(16)
- **COORDINATE:** Communication between care team members and care plan reconciliation(15)(17)(18)
- **COORDINATE:** Follow-up to evaluate substance misuse understanding, referral status, and need for further assistance(19)(20)
- **SEND:** Your care plan - Substance-related disorders  (Spanish)
- **SEND:** What you need to know - Substance-related disorders  (Spanish)
- **SEND:** When to get help - Substance-related disorders  (Spanish)
- **SEND:** Daily care for your condition - Substance-related disorders  (Spanish)
- **SEND:** Be ready for your doctor visit - Substance-related disorders  (Spanish)
- **SEND:** Be ready with questions for your doctor - Substance-related disorders  (Spanish)
- **SEND:** Tips for good mental health  (Spanish)
- **Goal:** Withdrawal information better understood(5)
 - **Interventions:**
 - **ASSIST:** Identifying patient's goal priorities, willingness/ability to achieve goals, and preferences for communication and follow-up
 - **ASSIST:** Goal setting that is specific, measurable, achievable, relevant, and with target completion time (eg, short/long-term, prioritized)
 - **EDUCATE:** Withdrawal symptoms need medical or behavioral attention (eg, suicidal ideation, signs of relapse).
 - **EDUCATE:** Withdrawal treatments may include medication and supportive care (eg, nutrition).
 - **EDUCATE:** **Alcohol** withdrawal symptoms and related management: See Alcohol Misuse  CCG for more information.
 - **EDUCATE:** **Anabolic steroid** withdrawal through drug tapering. Side effects and related management:
 - Side effects of steroid withdrawal include drug craving, fatigue, anorexia, headaches, muscle aches, insomnia, reduced sex drive, mood swings, restlessness, depression, and suicidal ideation.
 - Medications may be prescribed for withdrawal symptoms (eg, antidepressants for depression, analgesics for pain).
 - **EDUCATE:** **Benzodiazepines** withdrawal symptoms begin 1 to 5 days after last dose and include anxiety, light or sound sensitivity, confusion, and seizures.(8)
 - **EDUCATE:** **Hallucinogen** and **inhalant** cessation does not cause withdrawal syndrome.(8)
 - **EDUCATE:** **Cannabis** withdrawal symptoms (eg, irritability, anxiety, insomnia, appetite disturbance, depression) and related management(21)
 - **EDUCATE:** **Sedative-hypnotics** withdrawal symptoms and related management(8):
 - Minor withdrawal symptoms include anxiety, insomnia, tremors, and nightmares.
 - Major withdrawal symptoms include grand mal seizures, psychosis, hyperreflexia, and death.
 - **EDUCATE:** **Nicotine** withdrawal symptoms (eg, weight gain, insomnia, irritability, anxiety, depression, restlessness, difficulty concentrating) and related management(8)
 - **EDUCATE:** **Opioid** withdrawal symptoms and related management: See Narcotic/Opioid Misuse Assessment  CCG for more information.
 - **EDUCATE:** **Stimulant** (eg, cocaine, methamphetamine) withdrawal symptoms and related management(8):
 - Stimulant withdrawal symptoms include drug craving, agitation and restlessness, fatigue, unpleasant dreams, slowed activity, and delayed depression.
 - Stimulant withdrawal impacts sleep quality and pattern; sleep hygiene techniques include routine sleep/wake times and sleep in dark room.
 - No medications approved by the US Food and Drug Administration for cocaine relapse prevention(22)
 - **COORDINATE:** Treatment program referral, if indicated(5)
 - **COORDINATE:** Follow-up contact to review understanding of substance-related disorder treatment options, referral status, and need for further assistance(19)(20)

P2. Substance-related disorder self-care needed

- **Goal:** Substance-related disorder self-care plan developed, implemented, and maintained(3)(23)(24) 
- **Interventions:**

- **ASSIST:** Identifying patient's goal priorities, willingness/ability to achieve goals, and preferences for communication and follow-up
- **ASSIST:** Goal setting that is specific, measurable, achievable, relevant, and with target completion time (eg, short/long-term, prioritized)
- **ASSIST:** Appointment scheduling(25)
- **ASSIST:** Self-care plan creation between patient and provider (eg, goals, management, treating symptoms, getting help):
 - Development of crisis plan(8)
 - Support system (eg, family, friends) identification, enlistment, and education
- **ASSIST:** Communication between multiple providers(26)
- **EDUCATE: Self-care plan information(5):**
 - Action plan (eg, managing cravings, reacting to triggers)
 - Adherence to plan necessary to achieve goals, and complications of nonadherence (eg, exacerbations, progression, comorbidities)
 - Behavioral health treatment (eg, cognitive behavioral therapy (CBT), individual therapy, family therapy)(27)
 - Crisis care plan: turn off distractions (eg, TV, radio), seek emergent care, call hotline, call police if escalates; caregivers advised to not argue(28)
 - Daily care (eg, take medication, symptom management, crisis plan)
 - Diet considerations (eg, supplements for nutritional deficiencies)(29)(30)
 - Exercise program within provider's recommendations(31) 
 - Functional changes care (eg, adaptations and support for ADL and IADL)
 - Health checks, diary, and action plans (eg, rescue kit use)(32)
 - Medication purpose, use, effects, and considerations
 - Relapse signs (eg, insomnia, decreased appetite, poor personal hygiene)
 - Risk factor management (eg, trigger avoidance)
 - Safety awareness and management (eg, burn prevention, fall prevention)(33)
 - Sleep hygiene (eg, regular pattern, calm sleep environment)(8)
 - Stress management (eg, relaxation and coping techniques)
 - Substance use avoidance
 - Support group participation
 - Tobacco cessation plan
 - Wellness checkups (eg, vaccinations, screenings)
- **EDUCATE: When to get emergency help(34):**
 - Suicidal or homicidal actions or plans(35)
 - Heart attack (eg, persistent chest pain; pain radiating to the arms, shoulders, neck, back, and/or jaw; cool clammy skin)(36)
 - Headache (eg, sudden severe, unrelieved by analgesics)
 - Respiratory impairment (eg, gasping for air, choking, cyanosis, chest pain, severe dyspnea)(37)
 - Crisis plan not working
 - Hallucinations or delusions, increased or new
 - Infection, severe (eg, high fever, profuse sweating, change in consciousness)(32)(38)
 - Psychotic behavior (eg, threatening)
 - Seizures, new or complicated (eg, seizure longer than 5 minutes, multiple seizures with loss of consciousness, head injury during seizure)(39)
 - Delirium tremens (DT), usually starts 48 to 72 hours after last drink (eg, confusion, disorientation, tachycardia, severe tremors, hallucinations, and seizures)(7)
 - Drug overdose or alcohol poisoning signs (eg, loss of consciousness, problems breathing, coma, low blood pressure or body temperature)
- **EDUCATE: When to call the provider for behavior changes(5):**
 - Appointments missed (eg, support group, provider)
 - Behavior changes at work/school (eg, not getting work done on time, interpersonal conflicts, tardiness, or not going)
 - Cognitive changes (eg, thinking problems, forgetfulness worsens)
 - Depression (eg, feeling sad, hopeless, overwhelmed)(40)(41)
 - Legal problems (eg, being arrested, stealing, financial difficulties)
 - Life event, major (eg, job loss, death of someone close)
 - Mood changes (eg, depression, anger, agitation)
 - Substance misuse (eg, increased use, use interferes with ADL/IADL)
 - Suspension or expulsion from school
- **EDUCATE: When to call the provider(34):**
 - Constipation
 - Falls
 - Fatigue, unrelenting

- Gastrointestinal distress (eg, nausea, vomiting, diarrhea, cramping)
- Headache, prolonged or unresponsive to treatment
- Medication problems (eg, side effects, access, administration)
- Medications newly ordered from other providers
- Pregnant or planning to get pregnant(13)
- Sexual problems
- Sudden change in general health (eg, memory loss, high blood pressure)
- Sweating excessively
- Tachycardia
- Visual changes, new-onset
- **COORDINATE:** Behavioral health services referral, if indicated(10)
- **COORDINATE:** Psychiatrist referral, if indicated
- **COORDINATE:** Laboratory testing, if indicated
- **COORDINATE:** Community services referral, as needed(42)
- **COORDINATE:** Crisis services referral, as needed
- **COORDINATE:** School or education support, as needed
- **COORDINATE:** Social worker referral, as needed(14)
- **COORDINATE:** Support group referral, as needed
- **COORDINATE:** Telehealth referral, if indicated[E]
- **COORDINATE:** Tobacco use treatment, as needed
- **COORDINATE:** Care team meeting, if needed(15)(16)
- **COORDINATE:** Communication between care team members and care plan reconciliation(15)(17)(18)
- **COORDINATE:** Follow-up contact with patient to evaluate understanding and adherence to self-care plan, referral status, and need for further assistance(19)(20)
- **SEND:** Your care plan - Substance-related disorders  (Spanish)
- **SEND:** What you need to know - Substance-related disorders  (Spanish)
- **SEND:** When to get help - Substance-related disorders  (Spanish)
- **SEND:** Daily care for your condition - Substance-related disorders  (Spanish)
- **SEND:** Be ready for your doctor visit - Substance-related disorders  (Spanish)
- **SEND:** Be ready with questions for your doctor - Substance-related disorders  (Spanish)
- **SEND:** Important healthcare phone numbers  (Spanish)
- **SEND:** Caregiver tips  (Spanish)
- **SEND:** Choosing a place to live  (Spanish)
- **SEND:** Helping people who cannot think clearly  (Spanish)
- **SEND:** Help with sleeping or insomnia  (Spanish)
- **SEND:** How to stop smoking  (Spanish)
- **SEND:** Safety in the home - Fire and other risks  (Spanish)
- **SEND:** Tips for good mental health  (Spanish)
- **Goal:** Substance-related relapse control achieved and maintained
 - **Interventions:**
 - **ASSIST:** Identifying patient's goal priorities, willingness/ability to achieve goals, and preferences for communication and follow-up
 - **ASSIST:** Goal setting that is specific, measurable, achievable, relevant, and with target completion time (eg, short/long-term, prioritized)
 - **ASSIST:** Appointment scheduling
 - **EDUCATE:** Relapse signs starting:
 - Emergency help if overdose symptoms[F]
 - Sponsor contact
 - Support group contact and meeting attendance
 - Treatment program return
 - **EDUCATE:** Relapse risk and prevention techniques(43)(44):
 - Avoid activities with current users or where risk for use is increased.
 - Avoid triggers (stressors).
 - Seek help early if at-risk behaviors develop (eg, skipping or stopping medication, consumption of intoxicating substances).
 - Attendance at community support group and contact with sponsor if desire to use occurs(5)
 - Stress management (eg, relaxation and coping techniques)
 - Exercise program within provider's recommendations(45)

- Promote participation in activities that avoid substance use.(5)
- **EDUCATE:** Caregivers on interventions to support treatment(46):
 - Provide care during withdrawal, if indicated.(5)
 - Remove substances of misuse from patient's access.
 - Screen urine for drug use, if appropriate.(46)
 - Keep only immediately necessary dosage of medication within reach.
 - Benefit of support meeting attendance for caregiver and patient
 - Encourage sponsor contact.
- **EDUCATE:** Caregivers on signs of substance misuse(8):
 - **Alcohol:** Red eyes, slurred speech, flushed face, loss of muscle control, anorexia, weight loss, sleeping more, school or job performance declining, more absences from work or school, job loss, home responsibilities ignored, increased risky behavior, continued use despite personal or social losses(5)
 - **Anabolic steroids:** Paranoia, jealousy, extreme irritability, delusions, impaired judgment, and potentially violent ("roid rage")
 - **Cannabis:** Glassy red eyes, loud talking, inappropriate laughter, sleepiness, loss of interest or motivation, weight gain or loss
 - **Depressants:** Constricted pupils, difficulty concentrating, clumsiness, poor judgment, slurred speech, sleepiness
 - **Hallucinogens:** Dilated pupils, bizarre behavior, aggression, hallucinations, mood swings, isolation, slurred speech, confusion
 - **Inhalants:** Watery eyes; impaired vision, memory, or thought; nasal secretions or rash; headache; nausea; appearance of intoxication; drowsiness; poor muscle control; changes in appetite; anxiety; irritability; numerous cans/aerosols in trash
 - **Opioids:** Constricted pupils, needle marks, sleeping at unusual times, vomiting, coughing, twitching, anorexia, sweating
 - **Stimulants:** Dilated pupils, hyperactivity, euphoria, irritability, anxiety, mood swings, long time periods between eating or sleeping, weight loss, dry mouth or nose
- **COORDINATE:** Support group referral, as needed
- **COORDINATE:** Treatment program referral, if indicated
- **COORDINATE:** Vocational rehabilitation referral, as needed(13)
- **COORDINATE:** Social worker referral, as needed(14)
- **COORDINATE:** Care team meeting, if needed(15)(16)
- **COORDINATE:** Communication between care team members and care plan reconciliation(15)(17)(18)
- **COORDINATE:** Follow-up contact with patient to evaluate understanding and adherence to self-care plan, referral status, and need for further assistance(19)(20)
- **SEND:** Your care plan - Substance-related disorders  (Spanish)
- **SEND:** Daily care for your condition - Substance-related disorders  (Spanish)
- **SEND:** When to get help - Substance-related disorders  (Spanish)
- **SEND:** Caregiver tips  (Spanish)
- **SEND:** Help with sleeping or insomnia  (Spanish)

P3. Substance-related disorders potential complications and comorbidities monitoring needed

- **Goal:** Substance-related disorders potential complications and comorbidities evaluated and addressed(3)
- **Interventions:**
 - **ASSIST:** Identifying patient's goal priorities, willingness/ability to achieve goals, and preferences for communication and follow-up
 - **ASSIST:** Goal setting that is specific, measurable, achievable, relevant, and with target completion time (eg, short/long-term, prioritized)
 - **ASSIST:** Appointment scheduling for evaluation of problems, comorbidities, and self-care plan[G]:
 - Recently hospitalized patient's evaluation within 1 week of discharge
 - Recently hospitalized patient's evaluation with outpatient healthcare providers (eg, nurse, social worker) within 1 to 3 days of discharge(19)
 - Urine screening follow-up during medication tapering, if indicated(46)
 - Appointment every 3 to 6 months when patient is symptom free
 - Appointment once a month or more for patient with questionable adherence to treatment plan
 - **EDUCATE:** When and how to get help for worsening substance-related disorder problems
 - **EDUCATE:** Treatment and monitoring for comorbid conditions (eg, coronary artery disease, high blood pressure, heart valve disease)
 - **EDUCATE:** Perioperative pain management planning if taking medications for substance misuse (eg, buprenorphine, naltrexone)[H](47)(48)
 - **EDUCATE:** Complications of substance-related disorders:
 - Alcoholism complications (eg, cirrhosis, esophagitis, cardiovascular disease)(5)

- Risk of infection (eg, bacterial, fungal) from injection drug use(38)
- Smoking complications (eg, cancer, cardiovascular disease, pulmonary disease)(8)
- Substance misuse complications (eg, chronic pain, arrhythmias, endocarditis)(46)
- Steroid misuse complications (eg, kidney failure, liver damage, cardiovascular disease)(8)
- **EDUCATE:** Potential drug interactions with substance-related disorder medication and comorbid conditions
- **EDUCATE:** Vaccinations available for hepatitis A and B related to high-risk lifestyle (eg, injectable street drugs and unprotected sexual activities)
- **EDUCATE:** Perinatal effects of substance-related disorder and medication(13)(49)(50)
- **EDUCATE:** Follow-up appointments for assessment of clinical status
- **COORDINATE:** Specialist referral, if indicated
- **COORDINATE:** Treatment program referral, if indicated(51)
- **COORDINATE:** Preconception specialist referral, as needed
- **COORDINATE:** Care team meeting, if needed(15)(16)
- **COORDINATE:** Communication between care team members and care plan reconciliation(15)(17)(18)
- **COORDINATE:** Follow-up contact to review self-care plan and need for further assistance(19)(20)
- **SEND:** Be ready for your doctor visit - Substance-related disorders  (Spanish)
- **SEND:** Be ready with questions for your doctor - Substance-related disorders  (Spanish)
- **SEND:** Narcotic medicine side effects  (Spanish)
- **SEND:** Pain medicines - Addiction and withdrawal  (Spanish)
- **SEND:** When you are pregnant - Substance-related disorders  (Spanish)
- **SEND:** What you need to know - Hepatitis B  (Spanish)
- **SEND:** What you need to know - Hepatitis C  (Spanish)
- **SEND:** Hepatitis A vaccine  (Spanish)
- **SEND:** Hepatitis B vaccine  (Spanish)

P4. Substance-related disorder medication evaluation needed

- **Goal:** Substance-related disorder medication evaluation completed and medication regimen initiated, if indicated
 - **Interventions:**
 - **ASSIST:** Identifying patient's goal priorities, willingness/ability to achieve goals, and preferences for communication and follow-up
 - **ASSIST:** Goal setting that is specific, measurable, achievable, relevant, and with target completion time (eg, short/long-term, prioritized)
 - **ASSIST:** Appointment scheduling with provider for evaluation of condition status
 - **EDUCATE:** When and how to get help for worsening substance-related disorder problems
 - **EDUCATE:** Medication management needs common for substance-related disorders
 - **COORDINATE:** Follow-up after provider contact for evaluation of medication changes(19)(20)

P5. Substance-related disorder medication management needed

- **Goal:** General substance-related disorder medication information understood(52)
 - **Interventions:**
 - **ASSIST:** Identifying patient's goal priorities, willingness/ability to achieve goals, and preferences for communication and follow-up
 - **ASSIST:** Goal setting that is specific, measurable, achievable, relevant, and with target completion time (eg, short/long-term, prioritized)
 - **ASSIST:** Appointment scheduling with provider for discussion on appropriate medication
 - **ASSIST:** Providing reconciled medication list to all providers. See Medication Reconciliation Tool  SR for more information.
 - **EDUCATE: Purpose:** Importance of understanding what each medication is for, how it should be taken, and what to do if dose is missed
 - **EDUCATE:** Medication indications, side effects, and considerations frequently change. See a drug reference for specific information not listed here.
 - **EDUCATE:** Administration techniques:
 - Route (eg, oral, intramuscular (IM), intravenous (IV))
 - With or without foods or liquids (eg, milk)
 - Timing of administration
 - Appropriate measuring device (eg, liquid medications)
 - Proper storage
 - **EDUCATE: Get emergency help:** Adverse or allergic reactions (eg, wheezing, chest tightness, fever, itching, bad cough, blue skin color, hemorrhage, seizures, swelling of face, lips, tongue, or throat)
 - **EDUCATE: Side effects:** Management, and when to contact provider

- Common side effects include gastrointestinal changes, dry mouth, fatigue, insomnia, rash, dizziness, and headache.
- **EDUCATE: Considerations:**
 - Discuss medication with provider if pregnant, planning a pregnancy, or breast-feeding.
 - Comorbidities may be contraindications to the use of a drug (eg, liver disease, diabetes).
 - Health checks, diary, and action plans (eg, blood pressure, weight, heart rate)
 - Safety measures (eg, sun protection, driving, or operating machinery)
 - Contact provider or pharmacist prior to taking any other over-the-counter or prescribed medication.
 - Monitor closely for suicidal ideation and adverse effects, particularly early in treatment.
 - Discontinue medications under provider supervision only, as withdrawal symptoms may occur.
 - Continue taking medication even if feeling better.
 - Rescue kit use (eg, naloxone) as prescribed[F]
- **COORDINATE:** Clinical monitoring appropriate to medication use (eg, laboratory tests)
- **COORDINATE:** Communication between patient and pharmacy (for obtaining prescriptions, setting up home medication delivery, review for drug-drug interactions)[N]
- **COORDINATE:** Communication between care team members and medication list reconciliation
- **COORDINATE:** Follow-up to evaluate adherence or problems with medication, provide additional education and support, and identify need for further assistance(19)(20)
- **COORDINATE:** Laboratory testing, if indicated
- **COORDINATE:** Outpatient imaging, tests, or procedures, if indicated
- **COORDINATE:** Therapeutic drug level monitoring
- **SEND:** Managing your medicines (Illustrated) [F] (Spanish)
- **Goal:** Acamprosate initiated and maintained[N]
- **Interventions:**
 - **ASSIST:** Appointment scheduling with prescribing provider for medication evaluation and refills
 - **EDUCATE: Purpose:** Acamprosate decreases mild symptoms of protracted withdrawal (eg, insomnia, anxiety, restlessness, and dysphoria).
 - Taken 3 times per day
 - **EDUCATE: Side effects:** Anorexia, itching, weakness, anxiety, insomnia, sweating
 - **EDUCATE: Considerations:** Avoid in patients with moderate renal impairment.
- **Goal:** Anticonvulsant medication initiated and maintained(41)(53)[N]
- **Interventions:**
 - **ASSIST:** Appointment scheduling with prescribing provider for medication evaluation and refills
 - **EDUCATE: Purpose:** Anticonvulsants may be used as mood stabilizers or for seizure treatment.
 - **EDUCATE: Call provider immediately:** Vision problems, burning or tingling sensation, confusion, slowing of mental and physical ability, memory problems, speech problems, unusual fatigue, bleeding problems
 - **EDUCATE: Side effects:** Appetite changes, tremors
 - **EDUCATE: Considerations:**
 - Obtain baseline platelet count and bleeding time before starting valproic acid.
 - Valproic acid is not advisable for women of childbearing age due to the risk of birth defects.
 - **SEND:** Anticonvulsant medicines [F] (Spanish)
- **Goal:** Benzodiazepine initiated and maintained
- **Interventions:**
 - **ASSIST:** Appointment scheduling with prescribing provider for medication evaluation and refills
 - **EDUCATE: Purpose:** Benzodiazepine is first-line treatment for acute alcohol withdrawal symptoms.(54)
 - **EDUCATE: Call provider immediately:** Slurred speech; staggering; respiratory depression; severe depression, weakness, vertigo, confusion(55)
 - May cause overdose fatality when combined with opioids(13)
 - Alcohol use increases risk of respiratory depression and death.
 - **EDUCATE: Side effects:** Drowsiness, motor function impairment, urinary changes, confusion, vision changes, agitation(55)
 - **EDUCATE: Considerations:** Drug interactions may occur with herbal supplements (eg, St John's wort) and grapefruit, depending on specific benzodiazepine drug.
- **Goal:** Disulfiram initiated and maintained(5)(54)
- **Interventions:**
 - **ASSIST:** Appointment scheduling with prescribing provider for medication evaluation and refills
 - **EDUCATE: Purpose:** Disulfiram augments relapse prevention by producing autonomic nervous system instability (eg, flushing and palpitations) and vomiting in the presence of alcohol.
 - Effects of disulfiram will last up to 2 weeks after last dose.
 - **EDUCATE: Side effects:** Acne, fatigue, impotence, metallic or garlic-like taste in mouth
 - **EDUCATE: Considerations:** Read labels to avoid products containing alcohol (eg, cough syrup, mouthwash, perfume, aftershave, and certain extracts).

- **Goal:** Buprenorphine initiated and maintained(56)(57)(N)
 - **Interventions:**
 - **ASSIST:** Appointment scheduling with prescribing provider for medication evaluation and refills
 - **EDUCATE: Purpose:** Buprenorphine may be indicated for opioid dependence.
 - It helps to control drug cravings and drug-seeking behavior.
 - Can be combined with naloxone
 - **EDUCATE: Call provider immediately:** Respiratory depression
 - Dangerous side effects or death when combined with alcohol
 - **EDUCATE: Side effects:** Back pain, cough, fever, runny nose, insomnia(58)
 - Withdrawal syndrome milder, creating less dependence on drug
 - **EDUCATE: Considerations:**
 - Federal limitations to prescribing may apply based on specialty training.(13)
 - Treatment option for opioid-addicted pregnant women(13)
 - Treatment initiation requires abstinence from opioids. This varies from 12 to 72 hours based on the type of opioid (eg, short-acting, long-acting).
 - Monitor liver function.
- **Goal:** Methadone initiated and maintained(56)(57)
 - **Interventions:**
 - **ASSIST:** Appointment scheduling with prescribing provider for medication evaluation and refills
 - **EDUCATE: Purpose:** Methadone relieves opioid craving and suppresses the withdrawal syndrome without producing euphoric effects.(59)
 - Dose tapering: outpatient doses generally decreased by 10% to 20% every 1 to 2 days over 2 to 3 weeks(60)
 - **EDUCATE: Call provider immediately:** Respiratory depression
 - Dangerous side effects or death when combined with alcohol
 - **EDUCATE: Side effects:** Sweating, decreased sexual desire
 - **EDUCATE: Considerations:**
 - Treatment is restricted to federally regulated opioid treatment programs.(13)
 - Treatment option for opioid-addicted pregnant women(13)
- **Goal:** Naltrexone initiated and maintained(7)(61)
 - **Interventions:**
 - **ASSIST:** Appointment scheduling with prescribing provider for medication evaluation and refills
 - **EDUCATE: Purpose:** Naltrexone minimizes risk of relapses in alcohol and substance use disorders.
 - Blocks euphoric effects and reduces cravings(62)
 - **EDUCATE: Side effects:** Anxiety, nervousness, insomnia, flu-like symptoms, anorexia, fatigue
 - **EDUCATE: Considerations:**
 - Do not use unless abstinent from opioids for 7 to 10 days to avoid opioid withdrawal reaction.(57)
 - Preferred for patients who are highly motivated
 - Monitor liver function.

P6. Immediate personal harm (ie, suicide or harm to others) risk present

- **Goal:** Immediate behavioral health evaluation and intervention to prevent patient harming self or others
 - **Interventions:**
 - **ASSIST: If patient is actively threatening harm to self or others, contact the crisis hotline or emergency services while maintaining communication with the patient.**(63) Harming another person can be physical, emotional, or sexual.(64)
 - **ASSIST:** National Suicide Prevention Lifeline: 1-800-273-8255 or call, text, or live chat 988
 - **COORDINATE:** Follow-up contact after behavioral health evaluation to review self-care plan and identify need for further assistance(19)(20)

P7. Potential personal harm (ie, suicide or harm to others) risk present

- **Goal:** Personal harm (ie, suicide or harm to others) warning signs and action plan better understood
 - **Interventions:**
 - **ASSIST: If patient is actively threatening harm to self or others, contact the crisis hotline or emergency services while maintaining communication with the patient.**(63) Harming another person can be physical, emotional, or sexual.[]](64)
 - **ASSIST:** Identifying patient's goal priorities, willingness/ability to achieve goals, and preferences for communication and follow-up
 - **ASSIST:** Goal setting that is specific, measurable, achievable, relevant, and with target completion time (eg, short/long-term, prioritized)
 - **ASSIST:** Appointment scheduling for evaluation of behavioral health status
 - **ASSIST:** Identifying patient supports and setting up support system communication(63)
 - **ASSIST:** Creation of an emergency plan so family and caregivers know steps to take if personal harm symptoms present(63)

- **ASSIST:** Identifying risks of suicide or harm to self or others(65)(66)(67):
 - Self or family history of suicide or suicide attempts
 - Recent stressful event (eg, recent exposure to another's suicide) or serious adverse childhood event
 - History of being bullied
 - Social isolation or withdrawal
 - Severe functional impairments
 - Spousal loss
 - Discrimination experience (eg, lesbian, gay, bisexual, transgender)
 - Demographic factors (eg, male, older age, non-Hispanic White or American Indian/Alaska Native race)
 - Mental health disorders
 - Triggers include physical pain, loss of support or respect from family or peers, interpersonal conflicts, painful memories, loneliness, helplessness, lack of self-worth.
- **EDUCATE:** Behavioral health crisis care(65)(68):
 - Crisis care plan: turn off distractions (eg, TV, radio), seek emergent care, call hotline, call police if escalates; caregivers advised to not argue(35)
 - Call police if crisis involves violence.(35)
 - Keep the crisis hotline telephone number near the phone.
 - Keep list of names, addresses, and emergency telephone numbers for medical help and support individuals (eg, neighbor, relative, close friend).
- **EDUCATE:** Suicide warning signs(41)(69):
 - Depression (eg, feeling sad, hopeless, overwhelmed)(40)
 - Giving away personal possessions, settling accounts, or other actions relating to disposition of estate in case of death
 - Mood instability (eg, extreme or rapid changes)
 - Reckless activities
 - Sense of feeling trapped with no way out
 - Sleep pattern changes (eg, hypersomnia, insomnia)
 - Substance misuse (eg, increased use, use interferes with ADL/IADL)
 - Suicidal or homicidal actions or plans(35)
 - Social changes (eg, isolation, withdrawal)
- **EDUCATE:** Suicide prevention measures(70):
 - Take any hint of suicide seriously, and seek immediate professional help.
 - Encourage immediate intervention (eg, patient call crisis line, caregiver call provider).(41)
 - Keep list of names, addresses, and emergency telephone numbers for medical help and support individuals (eg, neighbor, relative, close friend).
 - Restrict access to methods of suicide.[J](41)
- **EDUCATE:** Suicidal ideation signs(41):
 - Expressing suicidal thoughts or actions, or potential for harm to self or others
 - Disclosing of suicide plan or plan to harm another
 - Warning signs for suicide attempt (eg, writing suicide note, giving away valued possessions)
 - Role playing (eg, standing on chair with noose)
 - Recurrent thoughts of death (not just fear of dying)
 - Available means to execute plan of harm to self or others
- **EDUCATE: When to get emergency help(41):**
 - Suicidal or homicidal actions or plans
 - Psychotic behavior (eg, threatening)
 - Crisis plan not working(71)
- **EDUCATE: When to call the provider(41)(72):**
 - Depression (eg, feeling sad, hopeless, overwhelmed)(40)(41)
 - Self-harm (eg, abuse, mutilation)
 - Hallucinations or delusions increase.
 - Cognitive changes (eg, thinking problems, forgetfulness worsens)
- **EDUCATE:** National Suicide Prevention Lifeline: 1-800-273-8255 or call, text, or live chat 988
- **COORDINATE:** Behavioral health services referral, if indicated(41)(63)
- **COORDINATE:** Psychiatrist referral, if indicated(41)(63)(73)
- **COORDINATE:** Community services referral, as needed
- **COORDINATE:** Protective services or legal authorities, as needed
- **COORDINATE:** Social worker referral, as needed(14)
- **COORDINATE:** Care team meeting, if needed(15)(16)
- **COORDINATE:** Communication between care team members and care plan reconciliation(15)(17)(18)
- **COORDINATE:** Follow-up contact to evaluate safety, referral status, and need for further assistance(19)(20)

P8. Depression evaluation needed

- **Goal:** Depression evaluated and self-care plan understood 
- **Interventions:**
 - **ASSIST:** Identifying patient's goal priorities, willingness/ability to achieve goals, and preferences for communication and follow-up
 - **ASSIST:** Goal setting that is specific, measurable, achievable, relevant, and with target completion time (eg, short/long-term, prioritized)
 - **ASSIST:** Appointment scheduling for symptoms of depression and self-care plan to include:
 - Support system (eg, family, friends) identification, enlistment, and education
 - Development of crisis plan
 - **EDUCATE: Definition:** Depression is a mood disorder characterized by persistent sadness and disinterest.(40)
 - **EDUCATE: Causes(41):**
 - Exact cause is unknown.
 - Risk factors (eg, life-changing event)
 - **EDUCATE: Symptoms(41):**
 - Appetite changes
 - Fatigue
 - Sleep problems
 - Concentration difficulty
 - Activity intolerance
 - Interest in activities or hobbies lost
 - **EDUCATE: Diagnosis:**
 - Based on medical history, risk factors, physical examination, and diagnostic tests
 - Behavioral health examination and history
 - **EDUCATE: Treatment(41):**
 - Self-care activities (eg, diet, activity, medication)
 - Comorbid condition management
 - Psychotherapy (eg, cognitive behavioral therapy (CBT), individual or group therapy, family therapy)
 - **EDUCATE: Treatment goals:**
 - Psychosocial functioning improved and maintained
 - Quality of life improvement
 - Recurrence prevention
 - Safety (for self and others)
 - Symptom frequency and severity reduced
 - **EDUCATE: When to get emergency help(74):**
 - Suicidal or homicidal actions or plans
 - Hallucinations or delusions, increased or new
 - Psychotic behavior (eg, threatening)
 - **EDUCATE: When to call the provider(41):**
 - Self-care changes (eg, ADL or IADL problems)
 - Mood changes (eg, depression, anger, agitation)
 - Cognitive changes (eg, thinking problems, forgetfulness worsens)
 - Depression (eg, feeling sad, hopeless, overwhelmed)(40)
 - Self-harm (eg, abuse, mutilation)
 - **EDUCATE: Resources:**
 - American Psychiatric Association: www.psychiatry.org
 - National Alliance on Mental Illness: www.nami.org
 - National Institute of Mental Health: www.nimh.nih.gov
 - Substance Abuse and Mental Health Services Administration: www.samhsa.gov
 - National Suicide Prevention Lifeline: www.988lifeline.org
 - **COORDINATE:** Behavioral health services referral, if indicated
 - **COORDINATE:** Psychiatrist referral, if indicated
 - **COORDINATE:** Crisis services referral, as needed
 - **COORDINATE:** Support group referral, as needed
 - **COORDINATE:** Care team meeting, if needed(15)(16)
 - **COORDINATE:** Communication between care team members and care plan reconciliation(17)(18)(20)
 - **COORDINATE:** Follow-up contact to evaluate depressive symptoms, referral status, and need for further assistance(19)(20)
 - **SEND:** What you need to know - Depression  (Spanish)
 - **SEND:** When to get help - Depression  (Spanish)
 - **SEND:** Be ready for your doctor visit - Depression  (Spanish)

- **SEND:** Be ready with questions for your doctor - Depression  (Spanish)
- **SEND:** Antidepressant medicines  (Spanish)
- **SEND:** When you are pregnant - Depression  (Spanish)
- **SEND:** Tips for good mental health  (Spanish)

P9. Substance misuse information needed

- **Goal:** Substance misuse information understood(8)(34)(75)
 - **Interventions:**
 - **ASSIST:** Identifying patient's goal priorities, willingness/ability to achieve goals, and preferences for communication and follow-up
 - **ASSIST:** Goal setting that is specific, measurable, achievable, relevant, and with target completion time (eg, short/long-term, prioritized)
 - **ASSIST:** Appointment scheduling for evaluation of substances used and impact on self-care plan
 - **EDUCATE:** Common substances that are misused
 - **EDUCATE:** Harm reduction measures (eg, not driving while intoxicated, communicable disease prevention)
 - **EDUCATE:** Risks associated with misusing substances (eg, health risks, relationship changes)
 - **EDUCATE:** Substance misuse or addiction signs (eg, behavior changes, drug seeking)(42)
 - **EDUCATE:** Withdrawal expectations
 - **EDUCATE:** When and how to get help if substances misused
 - **EDUCATE:** Complications if substance use not treated (eg, liver or renal failure)(43)
 - **COORDINATE:** Community services referral, as needed(44)
 - **COORDINATE:** Care team meeting, if needed(15)(16)
 - **COORDINATE:** Communication between care team members and care plan reconciliation(15)(17)(18)
 - **COORDINATE:** Follow-up contact to evaluate substance misuse understanding, referral status, and need for further assistance(19)(20)

P10. Substance use disorder potentially present

- **Goal:** Substance use reduced or stopped and effects managed(34)(42)
 - **Interventions:**
 - **ASSIST:** Identifying patient's goal priorities, willingness/ability to achieve goals, and preferences for communication and follow-up
 - **ASSIST:** Goal setting that is specific, measurable, achievable, relevant, and with target completion time (eg, short/long-term, prioritized)
 - **ASSIST:** Appointment scheduling for evaluation of substances used and impact on self-care plan
 - **ASSIST:** Appointment scheduling for behavioral health provider evaluation of new or worsening problems with substance use
 - **ASSIST:** Accessing benefits for substance use disorder programs (for medication or counseling)
 - **EDUCATE: Self-care plan information(5):**
 - Action plan (eg, managing cravings, reacting to triggers)
 - Symptom management (eg, withdrawal symptoms)
 - Daily care (eg, diet, exercise, medication)
 - Support person (eg, sponsor) help
 - Support group participation
 - Behavioral health treatment (eg, cognitive behavioral therapy (CBT), individual therapy, family therapy)(76)
 - Stress management (eg, relaxation and coping techniques)
 - Risk factor management (eg, trigger avoidance)
 - **EDUCATE: When to get emergency help(5):**
 - Drug overdose or alcohol poisoning (eg, loss of consciousness, problems breathing, coma, low blood pressure or body temperature)
 - Suicidal or homicidal actions or plans(35)
 - Delirium tremens (DT), usually starts 48 to 72 hours after last drink (eg, confusion, disorientation, tachycardia, severe tremors, hallucinations, and seizures)(7)
 - Headache (eg, sudden severe, unrelieved by analgesics)
 - Heart attack (eg, persistent chest pain; pain radiating to the arms, shoulders, neck, back, and/or jaw; cool clammy skin)(36)
 - Stroke (eg, face drooping, arm weakness, speech problems, bad headache, vision changes, loss of balance or coordination)(77)
 - **EDUCATE: When to call the provider for behavior changes(5)(43):**
 - Substance misuse (eg, increased use, use interferes with ADL/IADL)
 - Depression (eg, feeling sad, hopeless, overwhelmed)(40)(41)
 - Medication problems (eg, side effects, access, administration)
 - Legal problems (eg, being arrested, stealing, or financial difficulties)

- **EDUCATE: Resources:**
 - Cocaine Anonymous World Services: www.ca.org
 - Marijuana Anonymous World Services: www.marijuana-anonymous.org
 - Narcotics Anonymous: www.na.org
 - Substance Abuse and Mental Health Services Administration: www.samhsa.gov
- **COORDINATE: Substance-related disorders** assessment, if indicated(44)
- **COORDINATE:** Support group referral, as needed(5)
- **COORDINATE:** Behavioral health services referral, if indicated
- **COORDINATE:** Psychiatrist referral, if indicated(11)
- **COORDINATE:** Treatment program referral, if indicated
- **COORDINATE:** Social worker referral, as needed(14)
- **COORDINATE:** Community services referral, as needed(44)
- **COORDINATE:** Care team meeting, if needed(15)(16)
- **COORDINATE:** Communication between care team members and care plan reconciliation(15)(17)(18)
- **COORDINATE:** Follow-up contact to evaluate substance discontinuation, referral status, and need for further assistance(19)(20)
- **SEND:** Pain medicines - Addiction and withdrawal  (Spanish)
- **SEND:** When to get help - Substance-related disorders  (Spanish)
- **SEND:** Be ready for your doctor visit - Substance-related disorders  (Spanish)
- **SEND:** Be ready with questions for your doctor - Substance-related disorders  (Spanish)
- **SEND:** Tips for good mental health  (Spanish)

P11. Potential hazardous or at-risk alcohol use present

- **Goal:** Alcohol effects and complications of misuse understood(62)
- **Interventions:**
 - **ASSIST:** Identifying patient's goal priorities, willingness/ability to achieve goals, and preferences for communication and follow-up
 - **ASSIST:** Goal setting that is specific, measurable, achievable, relevant, and with target completion time (eg, short/long-term, prioritized)
 - **ASSIST:** Appointment scheduling for evaluation of effects of alcohol use and self-care plan, if needed
 - **EDUCATE:** Health effects of drinking(62)(78):
 - Light to moderate intake (2 drinks per day for men and 1 drink per day for women) may be linked with poor health outcomes.
 - Heavy drinking can cause acute alcohol poisoning and nutritional deficiencies.
 - Long-term effects of alcohol misuse include cirrhosis, pancreatitis, peripheral neuropathy, and cancer.(34)
 - **EDUCATE:** Follow drinking limits set by provider.
 - **EDUCATE:** Comparison of personal drinking to national norms(78):
 - Safe drinking levels for men (younger than 65 years) are 14 drinks or fewer per week and 4 drinks or fewer on one occasion.
 - Safe drinking levels for women are 7 drinks or fewer per week and 3 drinks or fewer on one occasion.
 - Safe drinking levels for healthy adults age 65 years are 7 drinks or fewer per week and 3 drinks or fewer on one occasion.
 - No drinking during pregnancy is safe. Caution should be used when breast-feeding; consumption of a single drink may be safe if the mother waits 4 hours before breast-feeding or expresses breast milk for feeding prior to consuming the alcoholic drink.
 - **EDUCATE:** Harmful alcohol interactions with medications (eg, with analgesics, antibiotics, anticoagulants, antidepressants, antihistamines, anticonvulsants, medications for diabetes, beta-blockers, and sedative/hypnotic medications)
 - **EDUCATE:** Role of self-help programs (eg, Alcoholics Anonymous) and cognitive behavioral therapy(44)
 - **EDUCATE: When to get emergency help(5):**
 - Drug overdose or alcohol poisoning (eg, loss of consciousness, problems breathing, coma, low blood pressure or body temperature)
 - Delirium tremens (DT), usually starts 48 to 72 hours after last drink (eg, confusion, disorientation, tachycardia, severe tremors, hallucinations, and seizures)(7)
 - **EDUCATE: When to call the provider for behavior changes:**
 - Substance misuse (eg, increased use, use interferes with ADL/IADL)
 - Medication problems (eg, side effects, access, administration)
 - Depression (eg, feeling sad, hopeless, overwhelmed)(40)(41)
 - **EDUCATE: Resources:**
 - Al-Anon Family Groups: www.al-anon.org
 - Alcoholics Anonymous World Services: www.aa.org

- National Institute on Alcohol Abuse and Alcoholism: www.niaaa.nih.gov
- Substance Abuse and Mental Health Services Administration: www.samhsa.gov
- **COORDINATE:** Behavioral health services referral, if indicated
- **COORDINATE:** Community services referral, as needed(44)
- **COORDINATE:** Support group referral, as needed
- **COORDINATE:** Social worker referral, as needed(14)
- **COORDINATE:** **Substance-related disorders** assessment, if indicated(44)
- **COORDINATE:** Care team meeting, if needed(15)(16)
- **COORDINATE:** Communication between care team members and care plan reconciliation(15)(17)(18)
- **COORDINATE:** Follow-up contact to evaluate alcohol use status, referral status, and need for further assistance(19)(20)
- **SEND:** Tips for good mental health  (Spanish)
- **SEND:** What you need to know - Substance-related disorders  (Spanish)
- **SEND:** Be ready for your doctor visit - Substance-related disorders  (Spanish)
- **SEND:** Be ready with questions for your doctor - Substance-related disorders  (Spanish)

P12. Social support needed

- **Goal:** Social support improved(81)(82) 
- **Interventions:**
 - **ASSIST:** Identifying patient's goal priorities, willingness/ability to achieve goals, and preferences for communication and follow-up
 - **ASSIST:** Goal setting that is specific, measurable, achievable, relevant, and with target completion time (eg, short/long-term, prioritized)
 - **ASSIST:** Patient in establishment of a helpful social network(83):
 - Identification of individuals in support network
 - Engaging in social support system (gathering information, helping patient develop skills, encouraging shifts in perspective, and allowing patient to vent about issues)
 - **EDUCATE:** Ways to access social support
 - **EDUCATE:** Role of social support in quality of life (eg, improved self-image, coping)(82)
 - **EDUCATE:** Patient's role in social support relationships (eg, be aware of how actions impact the feelings of family and friends)(84)
 - **COORDINATE:** Behavioral health services referral, if indicated
 - **COORDINATE:** Community services referral, as needed(85)
 - **COORDINATE:** Social worker referral, as needed(14)
 - **COORDINATE:** Support group referral, as needed(86)(87)
 - **COORDINATE:** Support network contact (eg, church, social groups), as needed(83)
 - **COORDINATE:** Follow-up to evaluate social support, referral status, and need for further assistance(19)(20)

P13. Healthy diet information needed

- **Goal:** Healthy diet better understood(88)(89) 
- **Interventions:**
 - **ASSIST:** Identifying patient's goal priorities, willingness/ability to achieve goals, and preferences for communication and follow-up
 - **ASSIST:** Goal setting that is specific, measurable, achievable, relevant, and with target completion time (eg, short/long-term, prioritized)
 - **ASSIST:** Appointment scheduling with provider for dietary review and self-care plan
 - **EDUCATE:** Healthy body weight and body mass index (BMI) for patient(90):
 - BMI equals body weight in kilograms divided by the square of the height in meters (kg/m²).
 - BMI of 18.5 to 24.9 is a normal weight.
 - BMI of 25 to 29.9 is overweight.
 - BMI of 30 or greater is obese.
 - **EDUCATE:** Healthy diet general recommendations(91)(92):
 - Well-balanced diet (ie, high in fruits and vegetables, low in sodium, fats, and sugars)
 - Drink enough fluids.
 - Safe eating habits (eg, do not skip meals)
 - Calorie amount appropriate (eg, factors: age, gender, activity level)
 - **EDUCATE:** Adequate fluid intake^[K](92)(93):
 - Amount of fluid intake needed depends on patient attributes, including medical condition, level of physical activity, and exposure to heat. 
 - Men age 19 years and older: 15.6 cups (3.7 L) per day from all sources (eg, food and fluids)
 - Women age 19 years and older: 11.4 cups (2.7 L) per day from all sources (eg, food and fluids)
 - **COORDINATE:** Dietitian referral, if indicated(94)

- **COORDINATE:** Community services referral, as needed
- **COORDINATE:** Follow-up contact with patient to evaluate diet understanding and adherence, referral status, and need for further assistance(19)(20)
- **SEND:** Healthy eating (Illustrated)  (Spanish)
- **SEND:** How to read food labels (Illustrated)  (Spanish)
- **SEND:** Weight diary (Illustrated)  (Spanish)
- **SEND:** Meal diary (Illustrated)  (Spanish)

P14. Potential nutritional problems

- **Goal:** Nutritional problems resolved(89)(95)
 - **Interventions:**
 - **ASSIST:** Identifying patient's goal priorities, willingness/ability to achieve goals, and preferences for communication and follow-up
 - **ASSIST:** Goal setting that is specific, measurable, achievable, relevant, and with target completion time (eg, short/long-term, prioritized)
 - **ASSIST:** Appointment scheduling with provider for dietary review and self-care plan
 - **ASSIST:** Creation of nutrition diary
 - **COORDINATE:** Laboratory testing, if indicated
 - **COORDINATE:** Calorie or fluid counting
 - **EDUCATE:** Nutritional recommendations for patient's health status(92)
 - **EDUCATE:** Food and drug interactions potentially impacting nutritional status
 - **COORDINATE:** Dietitian referral, if indicated(94)
 - **COORDINATE:** Home care referral, if indicated
 - **COORDINATE:** Pharmacy medication support, if indicated
 - **COORDINATE:** Rehabilitation therapy referral, if indicated
 - **COORDINATE:** Follow-up contact with patient to evaluate nutritional status, referral status, and need for further assistance(19)(20)
 - **SEND:** Eating to keep up your energy  (Spanish)
 - **SEND:** Eating when your stomach is upset  (Spanish)
 - **SEND:** Mouth sores or infections  (Spanish)
 - **SEND:** How to read food labels (Illustrated)  (Spanish)
 - **SEND:** Meal diary (Illustrated)  (Spanish)

P15. Food label reading information needed

- **Goal:** Food label understanding improved
 - **Interventions:**
 - **ASSIST:** Identifying patient's goal priorities, willingness/ability to achieve goals, and preferences for communication and follow-up
 - **ASSIST:** Goal setting that is specific, measurable, achievable, relevant, and with target completion time (eg, short/long-term, prioritized)
 - **ASSIST:** Appointment scheduling with provider, nurse, or dietitian to provide in-person instruction on how to read food labels
 - **EDUCATE:** Food labels contain the nutritional value of the product(96)(97):
 - Percent of daily values is usually based on a 2000-calorie (8374-kJ) diet.
 - Nutritional values are listed per one serving.
 - A package may have more than one serving in the whole package.
 - Nutritional value should be doubled, tripled, and so on based on the number of servings eaten.
 - Serving size varies from product to product.
 - Form of food has impact on nutrition (eg, increased sugar content of orange juice vs one orange).
 - **COORDINATE:** Dietitian referral, if indicated
 - **COORDINATE:** Follow-up contact to evaluate food label understanding, referral status, and need for further assistance(19)(20)
 - **SEND:** How to read food labels (Illustrated)  (Spanish)

P16. Tobacco use present

- **Goal:** Tobacco abstinence achieved(98) 
 - **Interventions:**
 - **ASSIST:** Identifying patient's goal priorities, willingness/ability to achieve goals, and preferences for communication and follow-up

- **ASSIST:** Goal setting that is specific, measurable, achievable, relevant, and with target completion time (eg, short/long-term, prioritized)
- **ASSIST:** Appointment scheduling with provider for evaluation for nicotine replacement therapy (NRT) or other medications to help patient quit using tobacco
- **ASSIST:** Developing a plan to quit(99):
 - Setting quit date(100)
 - Identifying reasons to quit (eg, add years to your life, for someone you love, save money, feel better)
 - Identifying techniques to quit (ie, "cold turkey," using nicotine replacement, or gradual withdrawal)(101)
 - Identifying coping mechanisms
- **ASSIST:** Identifying resources to help tobacco cessation
- **EDUCATE:** Impact of tobacco on life(34):
 - Current symptoms caused by tobacco use
 - Costs of tobacco use
 - Impact of tobacco use on home environment
 - Risks of continued smoking on health (eg, cancer, stroke, gum disease)
- **EDUCATE:** Benefits of tobacco cessation:
 - Within minutes to days - lower blood pressure, lower carbon monoxide levels, better stamina, and improved smell and taste
 - Within 2 to 4 weeks - less risk of respiratory infection
 - Within 1 to 3 months - improved lung function and circulation
 - Within 1 year - 50% reduction in risk for myocardial infarction(102)
 - Within 5 to 15 years - cardiovascular risk equal to that of someone who has never smoked
 - Within 10 years - lung cancer risk cut in half
- **EDUCATE:** Ways to stop smoking or using tobacco(34):
 - Get support (support group, counseling, family buy-in, written self-help materials).(103)NNNNN
 - Talk to providers for help.
 - Stress management (eg, relaxation and coping techniques)(100)
 - Nicotine replacement therapy (eg, patch, gum) or medications that may helpLNN
 - Healthy lifestyle (good nutrition, activity)(100)(104)N
- **EDUCATE:** Withdrawal symptoms that may be experienced (eg, trouble sleeping, replacing smoking urge with eating)
- **EDUCATE: Resources:**
 - American Lung Association: www.lung.org
 - Centers for Disease Control and Prevention: www.cdc.gov/tobacco
 - Smokefree. National Institutes of Health: www.smokefree.gov
- **COORDINATE:** Motivational counseling if patient not ready to quit(105)
- **COORDINATE:** Tobacco use treatment, as needed
- **COORDINATE:** Participation in stop-smoking programs (eg, American Cancer Society, American Lung Association)
- **COORDINATE:** Communication between care team members and care plan reconciliation(15)(17)(18)
- **COORDINATE:** Follow-up contact to monitor for withdrawal and evaluate tobacco use, referral status, and need for further assistance(19)(20)
- **SEND:** How to stop smoking 🇪🇸 (Spanish)
- **Goal:** Tobacco cessation medication treatment initiated and maintained(58)NN
 - **Interventions:**
 - **ASSIST:** Appointment scheduling with prescribing provider for medication evaluation and refills
 - **EDUCATE: Purpose:** Medication is used to reduce withdrawal feeling during smoking cessation.
 - Bupropion sustained-release and varenicline are first-line treatment, either alone or in combination.
 - Bupropion is recommended for 7 to 12 weeks. Continued treatment may be needed.
 - Varenicline can be used as maintenance therapy to prevent relapse.(106)
 - **EDUCATE: Call provider immediately:** Suicidal ideation, behavioral changes, or seizures(107)
 - **EDUCATE: Side effects:** Insomnia, tachycardia
 - **EDUCATE: Considerations:** Avoid bupropion in patients with seizure disorders.(58)
 - **SEND:** How to stop smoking 🇪🇸 (Spanish)
- **Goal:** Nicotine replacement therapy (NRT) initiated and maintained(58)
 - **Interventions:**
 - **ASSIST:** Appointment scheduling with prescribing provider for medication evaluation and refills
 - **EDUCATE: Purpose:** NRT is used to reduce withdrawal feeling during smoking cessation.
 - Dosage is individualized based on need and response of patient.
 - If using patch, apply one new patch every 24 hours on skin that is dry, clean, and hairless. Rotate skin sites. Do not cut patch, and do not use more than one patch at a time.
 - If using lozenge or gum, do not eat or drink for 15 minutes before or while utilizing.
 - **EDUCATE: Call provider immediately:** Nicotine toxicity (eg, nausea, abdominal pain, vomiting, diarrhea, sweating, disturbed vision or hearing, palpitations, altered respirations)

- **EDUCATE: Side effects:** Anxiety, sleep disorders, depression, myalgia
 - Patch may cause redness or swelling of the skin around the patch that does not go away.
- **EDUCATE: Considerations:** Avoid nicotine use in immediate post-MI period or in presence of serious arrhythmias or severe or worsening angina.
- **SEND:** How to stop smoking  (Spanish)

P17. Tobacco use in patient history

- **Goal:** Tobacco abstinence maintained(34)(108)
 - **Interventions:**
 - **ASSIST:** Identifying patient's goal priorities, willingness/ability to achieve goals, and preferences for communication and follow-up
 - **ASSIST:** Goal setting that is specific, measurable, achievable, relevant, and with target completion time (eg, short/long-term, prioritized)
 - **ASSIST:** Appointment scheduling with provider for evaluation of improved or stabilized health status, for reinforcement of abstinence from tobacco
 - **EDUCATE:** How to keep environment tobacco free
 - **EDUCATE:** Encourage continued abstinence from smoking and review tobacco cessation strategies.
 - **EDUCATE: Resources:**
 - American Lung Association: www.lung.org
 - Centers for Disease Control and Prevention: www.cdc.gov/tobacco
 - Smokefree. National Institutes of Health: www.smokefree.gov
 - **COORDINATE:** Tobacco use treatment, as needed
 - **COORDINATE:** Support group referral, as needed
 - **COORDINATE:** Communication between care team members and care plan reconciliation(15)(17)(18)
 - **COORDINATE:** Follow-up contact to evaluate tobacco use, referral status, and need for further assistance(19)(20)

P18. Vaccinations not up to date, increasing infection risk

- **Goal:** Vaccinations up to date(109)(110)
 - **Interventions:**
 - **ASSIST:** Identifying patient's goal priorities, willingness/ability to achieve goals, and preferences for communication and follow-up
 - **ASSIST:** Goal setting that is specific, measurable, achievable, relevant, and with target completion time (eg, short/long-term, prioritized)
 - **ASSIST:** Appointment scheduling
 - **ASSIST:** Identifying vaccines patient may need that are not contraindicated based on clinical status
 - **EDUCATE:** Purpose and benefits of vaccinations
 - **EDUCATE:** Recommended immunizations and timing of administration:
 - Centers for Disease Control and Prevention's vaccination schedule: www.cdc.gov/vaccines(111)
 - Vaccine schedules may need to be altered based on medications; check with the specialist provider for information.(112)
 - Possible side effects or complications associated with getting vaccinated
 - **EDUCATE:** Influenza vaccination annually(113):
 - Side effects are mild and last 1 or 2 days.
 - Common side effects with shot are sore, red, or swollen area where shot was given; fever; and aches.
 - Common side effects with nasal spray are mild and last 1 or 2 days; they include runny nose, headache, sore throat, and cough.
 - Allergic reaction rare and is usually caused by a chicken egg allergy
 - **EDUCATE:** Pneumococcal vaccination potential side effects:
 - Side effects are mild and last 1 or 2 days.
 - Common side effects include a sore, red, or swollen area where shot was given; fever; and muscle aches.
 - Serious side effects are rare.
 - **EDUCATE:** COVID-19 vaccination potential side effects(111):
 - Side effects are mild and last 1 or 2 days.
 - Common side effects include a sore, red, or swollen area where shot was given; chills; fatigue; and aches.
 - Serious side effects are rare.
 - **COORDINATE:** Community services referral, as needed
 - **COORDINATE:** Vaccinations
 - **COORDINATE:** Follow-up to evaluate vaccination understanding, referral status, and need for further assistance(19)(20)
 - **SEND:** Coronavirus disease 2019 (COVID-19) vaccines  (Spanish)
 - **SEND:** Flu vaccine  (Spanish)
 - **SEND:** Pneumonia vaccine for adults  (Spanish)

- **SEND:** Hepatitis A vaccine  (Spanish)
- **SEND:** Hepatitis B vaccine  (Spanish)

Evidence Summary

Quality standards: The Case Management Society of America's Standards of Practice for Case Management principles include facilitation of informed decision making, counseling, and health education with patient self-care that is responsive to the patient's preferences (eg, culture, values, needs).(114)

Quality standards: Standards indicate that program content should include an individualized self-care plan that includes patient preferences, prioritized goals, interventions with information on whom to contact in urgent situations, and scheduled follow-up. Programs should facilitate communication and collaboration with practitioners in a timely manner.(114)(115)(116)(117)

National guidelines: The 2018 Physical Activity Guidelines Advisory Committee advises that adults with chronic conditions or disabilities, who are able, should follow the key activity guidelines for adults. The guidelines recommend that adults should do at least 150 to 300 minutes a week of moderate-intensity or 75 to 150 minutes a week of vigorous-intensity aerobic physical activity, or an equivalent combination of moderate-intensity and vigorous-intensity aerobic activity. They should also do muscle-strengthening activities on 2 or more days a week. Older adults should do multicomponent physical activity that includes balance training as well as aerobic and muscle-strengthening activities.(118)

Quality standards: Standards indicate that programs should ensure that during care transitions the care plan is re-evaluated, modified, and communicated to appropriate care team members (eg, providers, caregivers) as needed to support the care transition.(114)(115)(116)(117)

Transition consideration: Several studies demonstrated a reduction in 30-day hospital readmissions when the patient follow-up was within 7 days of hospital discharge.(119)(120)(121) Earlier follow-up (eg, within 48 hours after hospital discharge) may also help reduce hospital readmissions for high-risk or vulnerable patients.(19) Follow-up appointment attendance rates were higher when appointments were scheduled before hospital discharge.(122)(123)

Outcomes: Several studies support the role of the pharmacist in medication reconciliation to identify and resolve pharmacy-related problems and improve outcomes (eg, prevention of medication-related adverse events, discontinuation of inappropriate medication).(124)

Evidence: Meta-analysis of alcohol dependence in adults has found that acamprosate may curtail alcohol use significantly when prescribed for durations of 3 to 12 months.(75)

Evidence: In a randomized clinical trial, gabapentin was found to be effective in treating alcohol dependence and relapse-related symptoms of insomnia, anxiety, dysphoria, cravings, headaches, and/or pain.(8)

Evidence: In a randomized controlled trial, patients who started taking gabapentin after 3 days of abstinence had fewer heavy drinking days (defined as 5 or more drinks for men and 4 or more drinks for women) and greater rates of abstinence than those who received a placebo.(125)

Evidence: A systematic review of 8 randomized controlled trials involving 709 participants found that maintenance treatment with buprenorphine appeared to be more effective than nonopioid treatment.(126)

National guidelines: The U.S. Preventive Services Task Force recommends screening for depression in patients 19 years and older combined with adequate systems to ensure treatment and follow-up; the task force concludes that such screening improves clinical outcomes (eg, reduction in depressive symptoms).(127)

Quality standards: Standards indicate that program content should address psychosocial issues (eg, depression) that may impede self-management. Programs should have a process for following up with patients on the status of referrals.(114)

Evidence: The Alcohol Use Disorders Identification Test (AUDIT) questionnaire's 3-item subset of questions about alcohol consumption (AUDIT-C) is a brief screening test for problem drinking that has been validated in primary care office settings.(78)

Quality standards: Standards indicate that program content should include a condition treatment plan, self-management and self-monitoring, and health behaviors (eg, substance use). Programs should have a process for following up with patients on the status of referrals.(114)

National guidelines: The US Department of Health and Human Services' Dietary Guidelines for Americans suggests that moderate alcohol consumption (2 drinks/day in men or 1 drink/day in women) may increase the overall risk of death from various causes, such as several types of cancer and some forms of cardiovascular disease.(91)

Specialty society: The American Heart Association does not support initiation of alcohol intake to improve cardiovascular health.(79)

Quality standards: Standards indicate that program content should address patient supports and psychosocial issues that may impede self-management. Programs should have a process for following up with patients on the status of referrals.(114)

National guidelines: The Institute of Medicine does not specify how much total fluid should come from water. Their recommendation is based on the median of total water intake on dietary surveys, which include water from liquids and solids. For healthy people, the advice is to consume at least 8 glasses (8 ounces (240 mL) each) of water daily; there is little evidence to support this recommendation.(128)

National guidelines: The U.S. Preventive Services Task Force (USPSTF) recommends that clinicians ask all adults about tobacco use, advise them to stop using tobacco, and provide behavioral interventions and FDA-approved pharmacotherapy for cessation to nonpregnant adults who use tobacco. The USPSTF recommends that clinicians ask all pregnant persons about tobacco use, advise them to stop using tobacco, and provide behavioral interventions for cessation to those who use tobacco.(129)

National guidelines: The U.S. Preventive Services Task Force (USPSTF) concludes that there is insufficient evidence to determine the balance of benefits and harms of primary care interventions for tobacco cessation among school-age children and adolescents who already smoke because of a lack of adequately powered studies on behavioral counseling interventions and a lack of studies on medications. Clinical judgment should be used to decide how to best help youth who use tobacco.(130)

Evidence: A systematic review of 33 trials with 21,600 participants found that incentives (eg, money, vouchers) improved smoking cessation rates at long-term follow-up (at least 6 months) in mixed-population studies. Ten trials of 2571 pregnant women found that women in the incentive-rewards group were more likely to stop smoking.(131)

Evidence: A systematic review of 47 studies with more than 18,000 people showed improved smoking cessation with increased therapeutic support and nicotine replacement therapy (NRT) as compared with NRT alone.(103)

Evidence: A review of 43 studies with more than 28,000 people found with moderate certainty that when no other support is available, written self-help materials help more people to stop smoking compared with getting no help at all.(132)

Evidence: A meta-analysis of 10 randomized studies of 3655 participants using nicotine replacement therapy found that behavioral support focusing on adherence to smoking cessation medications can modestly improve adherence.(133)

Evidence: A network meta-analysis of 33 systematic reviews with 250,563 participants found that behavioral support for smoking cessation can increase quit rates at 6 months or longer, with the strongest benefit for the provision of any type of counseling and guaranteed financial incentives.(134)

Evidence: A systematic review of 136 trials of nicotine replacement therapy (NRT) with 64,640 people found that all forms of NRT made it more likely that a person's attempt to quit smoking would succeed. Their chances of stopping smoking increased by 50% to 60% with or without additional counseling. There is no evidence that NRT increases the risk of heart attacks. In pregnant women, evidence suggests that NRT can increase the chance of quitting at the time of delivery.(135)

Evidence: A systematic review of 24 trials with 7279 participants found no evidence that adding exercise to smoking cessation support improves abstinence compared with support alone, but the evidence is insufficient to assess whether there is a modest benefit.(136)

Evidence: A systematic review of 51 studies including 22,000 people who smoked tobacco found that cessation was more likely if smokers cut down their intake when using stop-smoking medication (eg, varenicline) or fast-acting nicotine replacement therapy vs quitting all at once.(101)

Specialty Society: The American Thoracic Society recommendations for tobacco-dependent adults include using varenicline rather than a nicotine patch, using varenicline rather than bupropion, using varenicline rather than a nicotine patch in adults with a comorbid psychiatric condition, initiating varenicline in adults even if they are unready to quit, and using controller therapy for an extended treatment duration greater than 12 weeks.(106)

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Footnotes

[A] Assess and address the emergency or urgent problems before continuing the engagement. [A in Context Link 1]

[B] A self-care plan tells you how to take care of your condition and when and where to get help for problems. [B in Context Link 1]

[C] A standard drink consists of 0.6 ounces (18 mL) of ethanol, which is contained in 12 ounces (360 mL) of beer (alcohol by volume (ABV) = 5%), 5 ounces (150 mL) of wine (ABV = 12%), and 1.5 ounces (45 mL) of spirits (ABV = 40%).(91) [C in Context Link 1, 2]

[D] Medication management includes medically supervised withdrawal treatment (unless contraindicated).(5) [D in Context Link 1]

[E] Telehealth may improve access to and coordination of mental health care. Patients with behavioral health diagnoses may use telehealth or telepsychiatry to access mental health support or care (eg, to get assistance with problem-solving techniques or ask about presenting symptoms or medication use).(137) [E in Context Link 1]

[F] Naloxone rescue training and education about opioid overdoses should be offered to the patient and family/caregivers.(32) [F in Context Link 1, 2]

[G] The timing of provider follow-up depends on the patient's disease severity, self-care complexity, patient/caregiver willingness and ability to provide patient care, and available social supports.(138) [G in Context Link 1]

[H] For patients on chronic opioid therapy, perioperative planning should include multimodal pain-management techniques (eg, nonpharmaceutical interventions, nonopioid medications).(47) Buprenorphine and methadone for the treatment of opioid use disorder may be continued in the perioperative period for most patients.(47) For patients taking buprenorphine, a perioperative plan should be instituted, whether it is holding buprenorphine, continuing it perioperatively, or transitioning to another medication.(139) Patients taking methadone may take methadone on the day of surgery and continue a home maintenance dose through the perioperative period (confirm home maintenance dose with prescribing provider).(47) [H in Context Link 1]

[I] Patients with persistent thoughts of suicide, homicide, or serious harm to self or another may need frequent monitoring for progression to planning or intention to act on their thoughts. Inpatient treatment may be appropriate for people with depression who are at significant risk of suicide, self-harm, or self-neglect.(63) Other patients who will reliably participate in monitoring and have supports and providers available may be safely treated with frequent monitoring in outpatient care.(65)(68) [I in Context Link 1]

[J] Restricting access to suicide methods includes removing firearms or other weapons from the home, keeping only immediate medication dosages available, removing materials that could be used for hanging or suffocation, and access restrictions for jumping (eg, closed-caption video monitoring, physical barrier to bridges).(41)(69)(140)(141) [J in Context Link 1]

[K] Adequate intake is used when the recommended daily allowance cannot be determined because of insufficient data on requirements.(92) [K in Context Link 1]

[L] Primary pharmacotherapy for tobacco dependence includes varenicline, bupropion, nicotine gum, nicotine inhaler, nicotine nasal spray, and nicotine patch. Nicotine replacement therapies (NRTs) should be used with caution for patients in the 2 weeks following myocardial infarction and for those with life-threatening arrhythmias or severe angina.(98)(129) [L in Context Link 1]

Codes

ICD-10 Diagnosis: F10.10, F10.11, F10.120, F10.121, F10.129, F10.130, F10.131, F10.132, F10.139, F10.14, F10.150, F10.151, F10.159, F10.180, F10.181, F10.182, F10.188, F10.19, F10.20, F10.21, F10.220, F10.221, F10.229, F10.230, F10.231, F10.232, F10.239, F10.24, F10.250, F10.251, F10.259, F10.26, F10.27, F10.280, F10.281, F10.282, F10.288, F10.29, F10.90, F10.91, F10.920, F10.921, F10.929, F10.930, F10.931, F10.932, F10.939, F10.94, F10.950, F10.951, F10.959, F10.96, F10.97, F10.980, F10.981, F10.982, F10.988, F10.99, F11.10, F11.11, F11.120, F11.121, F11.122, F11.129, F11.13, F11.14, F11.150, F11.151, F11.159, F11.181, F11.182, F11.188, F11.19, F11.20, F11.21, F11.220, F11.221, F11.222, F11.229, F11.23, F11.24, F11.250, F11.251, F11.259, F11.281, F11.282, F11.288, F11.29, F11.90, F11.91, F11.920, F11.921, F11.922, F11.929, F11.93, F11.94, F11.950, F11.951, F11.959, F11.981, F11.982, F11.988, F11.99, F12.10, F12.11, F12.120, F12.121, F12.122, F12.129, F12.13, F12.150, F12.151, F12.159, F12.180, F12.188, F12.19, F12.20, F12.21, F12.220, F12.221, F12.222, F12.229, F12.23, F12.250, F12.251, F12.259, F12.280, F12.288, F12.29, F12.90, F12.91, F12.920, F12.921, F12.922, F12.929, F12.93, F12.950, F12.951, F12.959, F12.980, F12.988, F12.99, F13.10, F13.11, F13.120, F13.121, F13.129, F13.130, F13.131, F13.132, F13.139, F13.14, F13.150, F13.151, F13.159, F13.180, F13.181, F13.182, F13.188, F13.19, F13.20, F13.21, F13.220, F13.221, F13.229, F13.230, F13.231, F13.232, F13.239, F13.24, F13.250, F13.251, F13.259, F13.26, F13.27, F13.280, F13.281, F13.282, F13.288, F13.29, F13.90, F13.91, F13.920, F13.921, F13.929, F13.930, F13.931, F13.932, F13.939, F13.94, F13.950, F13.951, F13.959, F13.96, F13.97, F13.980, F13.981, F13.982, F13.988, F13.99, F14.10, F14.11, F14.120, F14.121, F14.122, F14.129, F14.13, F14.14, F14.150, F14.151, F14.159, F14.180, F14.181, F14.182, F14.188, F14.19, F14.20, F14.21, F14.220, F14.221, F14.222, F14.229, F14.23, F14.24, F14.250, F14.251, F14.259, F14.280, F14.281, F14.282, F14.288, F14.29, F14.90, F14.91, F14.920, F14.921, F14.922, F14.929, F14.93, F14.94, F14.950, F14.951, F14.980, F14.981, F14.982, F14.988, F14.99, F15.10, F15.11, F15.120, F15.121, F15.122, F15.129, F15.13, F15.14, F15.150, F15.151, F15.159, F15.180, F15.181, F15.182, F15.188, F15.19, F15.20, F15.21, F15.220, F15.221, F15.222, F15.229, F15.23, F15.24, F15.250, F15.251, F15.259, F15.280, F15.281, F15.282, F15.288, F15.29, F15.90, F15.91, F15.920, F15.921, F15.922, F15.929, F15.93, F15.94, F15.950, F15.951, F15.959, F15.980, F15.981, F15.982, F15.988, F15.99, F16.10, F16.11, F16.120, F16.121, F16.122, F16.129, F16.14, F16.150, F16.151, F16.159, F16.180, F16.183, F16.188, F16.19, F16.20, F16.21, F16.220, F16.221, F16.229, F16.24, F16.250, F16.251,

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